

ENGLISH FOR X-RAY TECHNICIANS

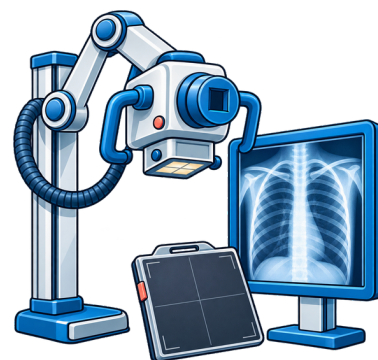
X-Ray & Radiography English

Verify the request. Guide the patient.
Close the handoff.

Phil Stilwell

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For X-ray technicians, radiologic technologists, and radiographers working within their authorized scope.

2026 edition

Eight scenarios. Eight skills.

Eight imaging encounters practice verification, stepwise instructions, sensitive questions, child-friendly language, bedside coordination, technical explanations, scope boundaries, and accurate incident reporting.

01 Resolving a request discrepancy

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The request says left; the patient says right | Pause an examination and verify conflicting information without suggesting an answer.

02 Adapting positioning instructions

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Explain the process without interpreting the image | Set a helpful professional boundary and give a concrete route for results.

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A clear instruction is part of a careful examination.

Radiography combines technical precision with short, consequential conversations. Patients need understandable instructions, an opportunity to report difficulty, and a clear distinction between acquiring images and interpreting them.

Verify before proceeding

An identity, examination, or laterality discrepancy requires the actual verification process. Do not resolve it by guessing from a schedule or silently changing a request.

"The request says left, but you describe the right side. I need to verify that discrepancy."

Make movement negotiable

Give one movement instruction at a time and check what the patient can do. Pain, mobility limitations, equipment, and consent may require an adapted approach.

"Tell me before moving if that position is painful or difficult."

Keep interpretation with the authorized clinician

Explain image acquisition and the reporting process within your role. A patient asking about a picture deserves a helpful handoff, not an improvised diagnosis.

"I can explain the examination; the reporting clinician interprets the images."

Preserve the record

Distinguish a technical limitation, a corrected record, and an incident. Use approved systems and escalation routes without deleting evidence or concealing an error.

"I have paused and preserved the record while the discrepancy is reviewed."

PROFESSIONAL BOUNDARIES

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

The request says left; the patient says right

A Read the situation

Pause an examination and verify conflicting information without suggesting an answer.

Morgan arrives for a wrist X-ray. The request names the left wrist, but Morgan describes an injury to the right wrist. Radiographer Alex has not acquired any images. Alex verifies identity using the approved identifiers, records the discrepancy, and contacts the requesting team through the department process. This case does not authorize Alex to rewrite the request.

PEOPLE / Alex: Radiographer | Morgan: Patient

ASK WITHOUT LEADING

Invite the patient to state information rather than simply agree with a suggested name or side. Explain the pause as a verification step, not as blame.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which side appears on the request?

A Neither

B Left

C Right

D Both

2. What does Morgan describe?

A A diagnosis from the image

B A right-wrist injury

C A left-ankle injury

D A completed examination

3. What happens before acquisition?

A Silently editing the request

B Imaging both sides automatically

C Verification through the department process

D Guessing the intended side

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

laterality

The specified right or left side of an examination.

IN USE verify laterality

imaging request

Authorized request describing the intended imaging examination.

IN USE review the imaging request

patient identifier

Approved identifying detail used to verify a person's identity.

IN USE confirm patient identifiers

examination site

Anatomical location intended for the examination.

IN USE verify the examination site

requesting clinician

Clinician who initiated the imaging request.

IN USE contact the requesting clinician

clinical indication

Clinical question or reason supporting an examination.

IN USE clarify the clinical indication

requisition

Document or electronic order requesting a service.

IN USE reconcile the requisition

worklist

Electronic list of scheduled or requested imaging examinations.

IN USE review the worklist

accession number

Identifier assigned to a particular imaging examination.

IN USE verify the accession number

identity discrepancy

Conflict between identifying details or records.

IN USE resolve an identity discrepancy

side marker

Marker identifying anatomical side on a radiographic image.

IN USE verify the side marker

pre-examination check

Verification performed before carrying out the examination.

IN USE complete the pre-examination check

Precision check

The patient account identifies a conflict; it does not independently authorize a different examination. Verification and request amendment are different actions.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

order clarification

Authorized clarification of an ambiguous or conflicting request.

IN USE obtain order clarification

acquisition

Process of obtaining the imaging data.

IN USE pause image acquisition

anatomical landmark

Identifiable body feature used to orient an examination.

IN USE identify an anatomical landmark

request amendment

Authorized documented change to an examination request.

IN USE record a request amendment

verification pause

Temporary stop while relevant information is checked.

IN USE explain a verification pause

resolved discrepancy

Conflict clarified through the appropriate documented process.

IN USE confirm a resolved discrepancy

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Use the approved identifiers and local procedure. An accession number identifies an examination, not a substitute for confirming the person.

Phrases you can actually use

C Notice the language / 1 of 2

INVITE THE ACCOUNT

"Please tell me which wrist was injured."

EXPLAIN THE PAUSE

"I need to clarify this before taking the images."

NAME THE CONTACT

"I will contact the team that requested the examination."

KEEP THE BOUNDARY

"I cannot silently change the request on that basis."

IDENTIFY THE CONFLICT

"The request names the left side, while you describe the right."

AVOID BLAME

"I am checking the information, not asking you to resolve the paperwork."

AVOID LEADING

"Please state your details rather than agreeing with my suggestion."

CONFIRM THE RESOLUTION

"We will proceed once the discrepancy is appropriately resolved."

While versus therefore

While can contrast two accounts; therefore would wrongly make the patient's account automatic authorization.

Before taking

Makes the verification step precede acquisition, rather than follow an avoidable exposure.

Names versus describes

Separates information in the request from the patient's own account.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which question avoids leading?

- A** It is your left wrist, correct?
- B** You mean left, do you not?
- C** Agree that the request is right.
- D** Please tell me which wrist was injured.

2. What does an accession number identify?

- A** The reporting diagnosis
- B** The painful side by itself
- C** A particular examination
- D** The patient's preferred language

3. Which action exceeds the supplied authorization?

- A** Silently changing the request
- B** Explaining the pause
- C** Contacting the requesting team
- D** Recording the conflict

4. Which wording preserves both sources?

- A** The request must be right.
- B** The patient must be wrong.
- C** Both sides are authorized.
- D** The request says left; the patient reports right.

[Answers and explanations: page 85.](#)

The request says left; the patient says right

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

accession number / requesting clinician / imaging request / patient identifier / laterality / verification pause / Acquisition / request amendment / clinical indication / resolved discrepancy

- 01 **Alex:** Before we start, please state the identifying details our department uses and tell me which wrist was injured.
- 02 **Morgan:** My right wrist. The receptionist mentioned the left, so I wondered whether something had been entered incorrectly.
- 03 **Alex:** The ¹ _____ does name the left wrist. I need to clarify that difference before taking any images.
- 04 **Morgan:** Can you just change it to right? That is definitely where I hurt myself yesterday.
- 05 **Alex:** Your account matters, but ² _____ needs verification through our process. I cannot silently rewrite the request.
- 06 **Morgan:** Does that mean you think I have given you the wrong information about my injury?
- 07 **Alex:** No. This ³ _____ checks the information from both sources. It is not a criticism of your account.
- 08 **Morgan:** Thank you. Who can clarify what examination was actually intended when the request was made?
- 09 **Alex:** I will contact the ⁴ _____ or the authorized team following our department procedure.
- 10 **Morgan:** I have a number on my appointment message. Would that settle which wrist you should examine?

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Alex:** The ⁵ _____ identifies the examination record. It does not resolve the side conflict on its own.
- 12 **Morgan:** I understand. You will still check that the record belongs to me before doing anything else?
- 13 **Alex:** Yes. We use the approved ⁶ _____ checks rather than relying only on an appointment number.
- 14 **Morgan:** What will you tell the team when you ask them to look at the request?
- 15 **Alex:** I will state the request's side, your account, and the relevant ⁷ _____ without inventing a new reason for imaging.
- 16 **Morgan:** And if they need to change the order, will that change be recorded somewhere?
- 17 **Alex:** Any authorized ⁸ _____ must follow the proper process so the record remains clear.
- 18 **Morgan:** All right. I would rather have the correct examination than hurry past a mismatch.
- 19 **Alex:** Exactly. ⁹ _____ has not started. I will explain the verified plan once the team responds.
- 20 **Morgan:** So we are waiting for a ¹⁰ _____, not assuming that either account can simply be ignored.

ANSWERS / EXPLANATIONS: PAGE 86

1. imaging request | 2. laterality | 3. verification pause | 4. requesting clinician | 5. accession number | 6. patient identifier | 7. clinical indication | 8. request amendment | 9. Acquisition | 10. resolved discrepancy

An ankle request with a missing side

F Say it

An ankle request omits the side. Jo reports a left-ankle injury. No images have been acquired, and the authorized team is being contacted.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

omits / acquired / left / clarify

Jo: "My injured ankle is the ¹ _____ one."

Radiographer: "The request currently ² _____ the side."

Jo: "The team will ³ _____ the request."

Radiographer: "No images have been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. left | 2. omits | 3. clarify | 4. acquired

Rehearse with a fixed script

1. Read the corrected exchange, stressing left and right without blaming either source.
2. Swap roles. Repeat the verification and authorization distinction without adding an examination.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

One movement at a time

A Read the situation

Give sequenced movement instructions and respond to a limitation without coercion.

Casey cannot comfortably raise the left shoulder for the planned position. Radiographer Jamie stops the attempted movement and checks the limitation. An adapted examination requires the appropriate departmental approach and, where needed, clinical advice. Casey has not refused all imaging; Casey has said this particular movement hurts. No alternative projection is prescribed by the exercise.

PEOPLE / Jamie: Radiographer | Casey: Patient

A LIMITATION IS USEFUL INFORMATION

Thank the patient for reporting pain. Avoid interpreting inability as unwillingness, and obtain agreement before offering physical assistance.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is difficult?

- A Raising the left shoulder
- C Hearing all instructions

- B Stating the name
- D Attending any imaging

2. What does Jamie do first?

- A Forces the position
- C Invents a new projection

- B Labels Casey uncooperative
- D Stops the attempted movement

3. What is not supplied?

- A Casey's reported pain
- C A need to adapt communication

- B A positioning difficulty
- D An authorized alternative projection

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

positioning

Arranging the patient's body for the intended image.

IN USE explain the positioning

projection

Image view defined by the direction of the X-ray beam.

IN USE clarify the requested projection

range of motion

Extent of movement available at a joint.

IN USE assess available range of motion

mobility limitation

Restriction affecting a person's movement.

IN USE accommodate a mobility limitation

weight bearing

Supporting body weight through a limb or joint.

IN USE clarify weight-bearing ability

transfer assistance

Help moving between surfaces or positions.

IN USE arrange transfer assistance

support aid

Equipment used to help maintain a suitable position.

IN USE select an approved support aid

upright position

Position with the upper body vertical.

IN USE explain an upright position

supine position

Position lying on the back.

IN USE describe the supine position

lateral position

Position lying on or oriented toward a side.

IN USE clarify the lateral position

rotation

Turning around a body or joint axis.

IN USE limit uncomfortable rotation

alignment

Relationship of body parts to the intended imaging geometry.

IN USE check patient alignment

Precision check

Cannot comfortably move is different from refuses imaging. Record the actual limitation and avoid labeling the patient uncooperative.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

immobilization

Restriction of movement using an appropriate method.

IN USE explain proposed immobilization

pain-limited movement

Movement restricted by the person's reported pain.

IN USE document pain-limited movement

physical assistance

Hands-on support provided with appropriate agreement and competence.

IN USE ask before physical assistance

breath-hold instruction

Direction to briefly suspend breathing for an image when appropriate.

IN USE rehearse a breath-hold instruction

adapted technique

Modified examination approach appropriate to the clinical and technical situation.

IN USE discuss an adapted technique

movement cue

Short instruction directing one specific movement.

IN USE give a clear movement cue

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A language exercise cannot determine a safe transfer, immobilization method, or alternative projection. Follow the actual assessment and authorized technique.

Phrases you can actually use

C Notice the language / 1 of 2

GIVE ONE STEP

"Please turn slightly toward me, then stop."

STOP CLEARLY

"Stop there; do not force that movement."

ASK BEFORE TOUCH

"May I help support your arm?"

EXPLAIN ADAPTATION

"I will check how we can adapt the examination appropriately."

INVITE A LIMIT

"Tell me before moving if that is painful or difficult."

CLARIFY ABILITY

"Which part of that movement is difficult?"

ACKNOWLEDGE THE REPORT

"Thank you for telling me about the pain."

REHEARSE FIRST

"We can practice the instruction before acquiring the image."

Cannot versus will not

Cannot describes a limitation; will not may imply refusal and should not replace the patient's actual account.

Then stop

Breaks a complex instruction into a bounded movement that can be checked.

May I help

Requests agreement before hands-on assistance rather than announcing it as inevitable.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which record is most accurate?

- A** Casey reports pain when raising the left shoulder.
- B** Casey refuses every examination.
- C** Casey is deliberately difficult.
- D** Casey has normal movement.

2. Which instruction respects the limitation?

- A** Keep going despite the pain.
- B** Ignore your shoulder.
- C** The picture matters more than your report.
- D** Stop there; do not force the movement.

3. What should precede hands-on help?

- A** An assumption of permission
- B** A surprise movement
- C** A guessed transfer method
- D** Appropriate agreement and assessment

4. What is a projection?

- A** A guaranteed alternative position
- B** A view defined by beam direction
- C** The patient's predicted diagnosis
- D** A consent signature

Answers and explanations: [page 87](#).

One movement at a time

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

adapted technique / range of motion / movement cue / projection / Physical assistance / support aid / positioning / breath-hold instruction / mobility limitation / pain-limited movement

- 01 **Jamie:** I will explain one movement at a time. Please tell me before moving if a position is painful or difficult.
- 02 **Casey:** Raising this left shoulder hurts. I can turn a little, but I cannot lift the arm that high.
- 03 **Jamie:** Stop there. This is ¹ _____, and we should not force the position just to follow the first instruction.
- 04 **Casey:** I do want the examination. I am worried you will think I am refusing to cooperate.
- 05 **Jamie:** I will record the actual ² _____, not describe you as refusing every part of the examination.
- 06 **Casey:** Thank you. It is easier when I know exactly which movement you are asking me to try.
- 07 **Jamie:** I can give a shorter ³ _____ and pause between steps. We can check what is comfortable before proceeding.
- 08 **Casey:** Could you explain what the position is intended to show without asking me to force it?
- 09 **Jamie:** The requested ⁴ _____ is a particular view. I will check the appropriate approach when that positioning is difficult.
- 10 **Casey:** Would a cushion or some support help, or does that need to be checked as well?

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Jamie:** Any ⁵ _____ must be appropriate for this examination.
- 12 **Casey:** I would also like you to ask before moving my arm, because unexpected movement makes me tense.
- 13 **Jamie:** Of course. ⁶ _____ needs your agreement and a suitable assessment. May I help support it while you stay still?
- 14 **Casey:** Yes, supporting it is comfortable. Please do not lift it farther without checking with me first.
- 15 **Jamie:** Understood. I will keep your ⁷ _____ limitation clear when I discuss the examination with the team.
- 16 **Casey:** Does changing the way I sit automatically mean you can take a different view instead?
- 17 **Jamie:** No. An ⁸ _____ must fit the clinical question and our authorized process, rather than be an improvised substitute.
- 18 **Casey:** Once that is settled, could we practice any breathing instruction before the image is taken?
- 19 **Jamie:** Yes, if a ⁹ _____ is appropriate, we can rehearse it and check that the wording is manageable for you.
- 20 **Casey:** Clear ¹⁰ _____ instructions and a pause give me a chance to tell you what I can actually do.

ANSWERS / EXPLANATIONS: PAGE 88

1. pain-limited movement | 2. mobility limitation | 3. movement cue | 4. projection | 5. support aid | 6. Physical assistance | 7. range of motion | 8. adapted technique | 9. breath-hold instruction | 10. positioning

A transfer needs assessment

F Say it

Drew says standing from the chair is difficult. The radiographer has not assessed transfer assistance and asks the appropriate team for help. Drew has not refused imaging.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

refused / assistance / difficult / assessment

Drew: "Standing from this chair is ¹ _____."

Radiographer: "The transfer needs ² _____."

Drew: "I have not ³ _____ the examination."

Radiographer: "I will request appropriate ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. difficult | 2. assessment | 3. refused | 4. assistance

Rehearse with a fixed script

1. Read the corrected exchange with a clear pause after each movement instruction.
2. Swap roles. Contrast a painful movement with refusal, and practice asking before assistance.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A private answer before the examination

A Read the situation

Explain a pregnancy-screening question respectfully and preserve an uncertain answer.

Rowan is asked a pregnancy-screening question under the department protocol and requests privacy from a companion. Rowan is unsure of the answer. Radiographer Taylor offers a private conversation and follows the appropriate review process. The uncertainty is not recorded as a negative answer. The exercise supplies neither an individualized radiation-risk estimate nor a decision to perform or cancel the examination.

PEOPLE / Taylor: Radiographer | Rowan: Patient

EXPLAIN WHY BEFORE ASKING

A personal question can feel intrusive when asked without context or in front of a companion. Offer appropriate privacy without making assumptions about identity, relationships, or reproductive history.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Rowan request?

- A A diagnosis from the image
- C An invented risk estimate

- B Automatic cancellation
- D Privacy from a companion

2. What answer is supplied?

- A Uncertainty
- C A verified negative result

- B Confirmed pregnancy
- D Refusal of all care

3. What decision is not supplied?

- A Whether to perform or cancel the examination
- C A privacy request

- B A need for review
- D A screening question

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

pregnancy screening

Process of checking relevant pregnancy information before an examination.

IN USE explain pregnancy screening

possible pregnancy

Pregnancy that has not been confirmed or excluded.

IN USE report possible pregnancy

screening uncertainty

Unresolved information from a screening question or process.

IN USE preserve screening uncertainty

private conversation

Discussion held away from people not authorized to hear it.

IN USE offer a private conversation

sensitive history

Personal information requiring respectful and appropriate handling.

IN USE obtain a sensitive history

ionizing radiation

Radiation capable of removing electrons from atoms.

IN USE explain ionizing radiation

radiation exposure

Exposure to radiation during an examination or other event.

IN USE discuss radiation exposure

justification

Assessment that an examination's expected benefit warrants its use.

IN USE review examination justification

optimization

Adjusting imaging practice to achieve its purpose with appropriate exposure.

IN USE explain optimization

benefit-risk discussion

Comparison of expected benefit and relevant possible harm.

IN USE arrange a benefit-risk discussion

radiologist review

Assessment by a physician specializing in medical imaging.

IN USE request radiologist review

examination protocol

Approved approach for a particular imaging examination.

IN USE follow the examination protocol

Precision check

Unsure is not no. Screening information supports the appropriate review; it does not create a universal instruction to cancel or proceed.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

screening response

Recorded answer to a screening question.

IN USE document the screening response

exposure history

Account of relevant prior radiation exposure.

IN USE clarify the exposure history

reproductive history

Relevant history concerning pregnancy and reproduction.

IN USE ask about reproductive history

privacy preference

Person's stated wishes about who hears personal information.

IN USE respect a privacy preference

clinical urgency

Degree to which the clinical situation requires prompt action.

IN USE communicate clinical urgency

review outcome

Decision or findings following the appropriate assessment.

IN USE explain the review outcome

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Risk depends on the actual examination and circumstances. Do not promise zero risk or claim that every X-ray is forbidden during pregnancy.

Phrases you can actually use

C Notice the language / 1 of 2

03

GIVE THE REASON

"This question is part of our examination-screening process."

ALLOW UNCERTAINTY

"It is all right to say that you are unsure."

AVOID A BLANKET RULE

"The appropriate team needs to review the actual examination."

PROTECT THE BOUNDARY

"I cannot give an individualized risk estimate from this information alone."

OFFER PRIVACY

"Would you prefer to discuss this without your companion present?"

RECORD ACCURATELY

"I will record uncertainty rather than a negative answer."

EXPLAIN THE REFERRAL

"I will arrange the relevant clinical discussion."

RETURN WITH A PLAN

"We will explain the reviewed plan before proceeding."

Unsure versus no

Preserves unresolved information instead of forcing a binary answer.

Actual examination

Keeps the discussion specific rather than applying a rule to every imaging procedure.

Would you prefer

Offers a privacy choice without assuming who should be present.

Phrases you can actually use

C Notice the language / 2 of 2

03

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

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"I will distinguish what was reported from what was observed."

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"We have agreed on the next step, not on an outcome."

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Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which entry matches Rowan's response?

- A Unsure
- B Negative
- C Confirmed positive
- D No screening required

2. Which statement overpromises?

- A I will preserve your answer.
- B There is absolutely no risk in every case.
- C The team will review this examination.
- D We can speak privately.

3. What should guide the next step?

- A An invented exposure calculation
- B The actual protocol and clinical review
- C A blanket cancellation rule
- D The companion's guess

4. Which phrase supports privacy?

- A I will announce the answer outside.
- B We can discuss this without your companion present.
- C Your companion must answer for you.
- D Everyone needs to hear this.

Answers and explanations: [page 89](#).

A private answer before the examination

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

private conversation / review outcome / benefit-risk discussion / Screening uncertainty / ionizing radiation / privacy preference / radiologist review / examination protocol / screening response / Justification

- 01 **Taylor:** I need to ask a question required by our screening process. Would you prefer to discuss it privately?
- 02 **Rowan:** Yes. My companion does not know everything about my situation, and I would rather answer without them here.
- 03 **Taylor:** We can arrange a ¹ _____. I will explain why the question matters before asking for personal information.
- 04 **Rowan:** Thank you. I may not be able to give a definite answer about whether I could be pregnant.
- 05 **Taylor:** Please say that clearly. ² _____ is important information; it should not be changed to no.
- 06 **Rowan:** Does being unsure automatically mean that every kind of X-ray has to be canceled?
- 07 **Taylor:** No blanket conclusion follows. The ³ _____ and clinical circumstances guide the appropriate review.
- 08 **Rowan:** I would like someone to explain the possible benefit and risk of this particular examination.
- 09 **Taylor:** I can arrange a ⁴ _____ with the appropriate professional, without inventing an individual estimate myself.
- 10 **Rowan:** I have heard the word radiation, but I do not know what kind this examination uses.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Taylor:** X-rays use ⁵ _____. The team can explain what that means for the proposed examination and your circumstances.
- 12 **Rowan:** Will my uncertain answer still be visible when the person reviewing the examination reads the record?
- 13 **Taylor:** Yes. The ⁶ _____ must reflect what you said, not a more convenient definite answer.
- 14 **Rowan:** Could the reason the examination is needed also matter to that discussion?
- 15 **Taylor:** Yes. ⁷ _____ concerns whether the expected benefit warrants the examination in its clinical context.
- 16 **Rowan:** And I would like my companion included only after we agree what can be discussed with them.
- 17 **Taylor:** I will respect that ⁸ _____ within the applicable rules and explain any relevant limits.
- 18 **Rowan:** Who will tell me what the review means for the next step?
- 19 **Taylor:** We will arrange the appropriate ⁹ _____ or other authorized clinical review, then explain the outcome and next step.
- 20 **Rowan:** That gives me a clear process without pretending the ¹⁰ _____ has already been decided.

ANSWERS / EXPLANATIONS: PAGE 90

1. private conversation | 2. Screening uncertainty | 3. examination protocol | 4. benefit-risk discussion | 5. ionizing radiation | 6. screening response | 7. Justification | 8. privacy preference | 9. radiologist review | 10. review outcome

A companion asks for the answer

F Say it

A companion asks what Rowan said. Rowan has requested privacy and has not authorized sharing the answer. The clinical review remains pending.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

authorized / pending / privacy / answer

Companion: "Can you share the screening ¹ _____?"

Radiographer: "Sharing has not been ² _____."

Companion: "The review is still ³ _____?"

Radiographer: "I need to respect the requested ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. answer | 2. authorized | 3. pending | 4. privacy

Rehearse with a fixed script

1. Read the corrected exchange. Keep unsure distinct from no throughout.
2. Swap roles. Explain privacy and review without supplying a risk estimate or examination decision.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A rehearsal before the picture

A Read the situation

Use concrete language, bounded choices, and honest reassurance with a child.

Eight-year-old Mina is anxious about a requested chest X-ray. Radiographer Lee explains the equipment and rehearses the relevant instruction before acquisition. Mina worries that the machine will touch her. Her caregiver is available, but any presence or assistance during exposure depends on the department safety process. The exercise does not prescribe restraint or a caregiver position.

PEOPLE / Lee: Radiographer | Mina: Child patient

INCLUDE THE CHILD DIRECTLY

Speak to the child rather than only about the child. A choice about explanation or rehearsal is meaningful; do not offer a false choice about a decision that has not been assessed.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What worries Mina?

- A Whether the machine will touch her
- C A report already issued

- B A known diagnosis
- D A prescribed restraint

2. What happens before acquisition?

- A Unapproved restraint
- C Explanation and rehearsal

- B Automatic caregiver exposure
- D A promise of no possible discomfort

3. What controls caregiver assistance?

- A A universal rule invented here
- C The appointment color

- B The child's guess
- D The department safety process

Answers and explanations: [page 91](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

age-appropriate explanation

Explanation adapted to the child's understanding and communication needs.

IN USE give an age-appropriate explanation

assent

Child's affirmative agreement appropriate to age and circumstances.

IN USE seek meaningful assent

caregiver permission

Authorization from an appropriate caregiver when required.

IN USE verify caregiver permission

child-life support

Specialist support helping children cope with healthcare experiences.

IN USE request child-life support

demonstration

Showing an action before asking someone to perform it.

IN USE offer a demonstration

practice run

Rehearsal before the actual procedure or acquisition.

IN USE complete a practice run

bounded choice

Limited genuine choice within the available safe options.

IN USE offer a bounded choice

sensory description

Explanation of what someone may see, hear, or feel.

IN USE provide a sensory description

distress cue

Sign suggesting anxiety, discomfort, or difficulty coping.

IN USE notice a distress cue

coping strategy

Method helping a person manage an uncomfortable experience.

IN USE agree on a coping strategy

caregiver presence

Caregiver being with the child during a relevant stage.

IN USE clarify caregiver presence

protective arrangement

Appropriate safety setup for people involved in an examination.

IN USE confirm protective arrangements

Precision check

Assent and caregiver permission are not interchangeable. Requirements depend on age, capacity, law, and setting; involve the child appropriately without inventing a legal rule.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

stillness

Absence of movement during the relevant acquisition interval.

IN USE explain brief stillness

exposure signal

Cue associated with the actual imaging exposure.

IN USE explain the exposure signal

reassurance

Supportive explanation that does not promise an unsupported outcome.

IN USE give honest reassurance

stop signal

Agreed cue for communicating a need to pause.

IN USE agree on a stop signal

cooperation

Participation supported by understanding and appropriate assistance.

IN USE support cooperation

preparation sequence

Ordered explanation and steps before the examination.

IN USE simplify the preparation sequence

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Avoid promises such as nothing can hurt or your caregiver can always stand here. Explain the actual equipment and approved arrangements honestly.

Phrases you can actually use

C Notice the language / 1 of 2

ADDRESS THE CHILD

"Mina, I will explain what happens before we start."

OFFER A REAL CHOICE

"Would you like me to explain first or demonstrate first?"

INVITE A QUESTION

"Which part worries you most?"

EXPLAIN CAREGIVER ARRANGEMENTS

"We need to check where your caregiver can safely be."

DESCRIBE HONESTLY

"I can show you which part makes the picture."

REHEARSE

"We can practice that instruction before taking the picture."

BOUND THE REASSURANCE

"I will tell you before we change position."

AGREE ON A PAUSE

"Let us agree how you can tell me you need a pause."

First or first

Offers a choice about the order of explanation, not a false choice about clinical authorization.

Before we start

Gives the child a predictable sequence and time for questions.

Can safely be

Makes caregiver placement conditional on the actual safety arrangements.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which choice is genuine here?

- A** Unlimited movement during acquisition
- B** Explanation first or demonstration first
- C** Any unassessed exposure position
- D** A guaranteed normal result

2. Which reassurance is honest?

- A** Your caregiver can stand anywhere.
- B** I will explain before we change position.
- C** Nothing can ever feel uncomfortable.
- D** Every result will be normal.

3. What does a practice run do?

- A** Replaces all permission requirements
- B** Establishes a diagnosis
- C** Guarantees cooperation
- D** Rehearses an instruction before acquisition

4. Which terms require distinction?

- A** Pause and brief stop
- B** Explain and clarify
- C** Assent and caregiver permission
- D** Picture and image in plain explanation

Answers and explanations: [page 91](#).

A rehearsal before the picture

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

sensory description / practice run / exposure signal / protective arrangement / assent / demonstration / age-appropriate explanation / preparation sequence / bounded choice / stop signal

- 01 **Lee:** Mina, I will explain the equipment before we begin. Which part of having the picture taken worries you most?
- 02 **Mina:** The big machine looks as though it might move toward me. I do not know what it does.
- 03 **Lee:** Let me give a ¹ _____ of this equipment, then show you the part used for the picture.
- 04 **Mina:** Could you show me first? It is easier when I can see what you mean.
- 05 **Lee:** Yes. That is a useful ² _____: demonstration first, then we can talk through the instructions.
- 06 **Mina:** Do I have to understand every long word before we can do the examination?
- 07 **Lee:** No. An ³ _____ uses words that make sense to you. You can ask me to explain anything again.
- 08 **Mina:** I heard someone say stay still. I am worried that I will forget what to do.
- 09 **Lee:** We can have a ⁴ _____ before the picture, so you know the sequence rather than guessing during it.
- 10 **Mina:** Will you tell me when the practice finishes and when the real picture begins?

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Lee:** Yes. I will explain the ⁵ _____ used here and distinguish practice from the actual picture.
- 12 **Mina:** Can my caregiver stay beside me the whole time? I feel calmer when I can see them.
- 13 **Lee:** We need to check the ⁶ _____ before promising where anyone can stand during exposure.
- 14 **Mina:** All right. Could you also tell me before asking me to move into a different position?
- 15 **Lee:** Certainly. A clear ⁷ _____ helps you know what happens next. I will explain before changing the instruction.
- 16 **Mina:** What should I do if I get frightened or do not understand the next step?
- 17 **Lee:** Let us agree on a ⁸ _____ so you can tell me. We will explain how to use it here.
- 18 **Mina:** I would like to practice with you before the picture. That feels less surprising.
- 19 **Lee:** Your ⁹ _____ matters. We also follow the required permission and safety process with your caregiver.
- 20 **Mina:** I understand. The ¹⁰ _____ first, then a practice, and I can tell you when I need something explained again.

ANSWERS / EXPLANATIONS: PAGE 92

1. sensory description | 2. bounded choice | 3. age-appropriate explanation | 4. practice run | 5. exposure signal | 6. protective arrangement | 7. preparation sequence | 8. stop signal | 9. assent | 10. demonstration

The instruction was too long

F Say it

Noah forgets a three-part instruction during practice. The radiographer breaks it into one step at a time. No image has yet been acquired.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

step / practice / taken / instruction

Noah: "I forgot the three-part ¹ _____."

Radiographer: "We can use one ² _____ at a time."

Noah: "This is still ³ _____."

Radiographer: "No image has been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. instruction | 2. step | 3. practice | 4. taken

Rehearse with a fixed script

1. Read the corrected exchange, using a warm but precise voice for the instructions.
2. Swap roles. Practice the real preparation choice without promising a result or caregiver position.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The bedside team stays connected

A Read the situation

Coordinate equipment, patient movement, and responsibilities before a bedside examination.

05

Radiographer Sam arrives for a portable chest examination. Nurse Priya identifies attached lines and monitoring equipment. Sam asks for the appropriate bedside team to coordinate any movement or equipment management. No line is disconnected, no device setting is changed, and no examination is performed until the actual preparation and safety checks are complete.

PEOPLE / Sam: Radiographer | Priya: Bedside nurse

STATE THE DEPENDENCY DIRECTLY

A polite request can still be explicit: identify the preparation needed and who is authorized to provide it. Do not treat clinical equipment as furniture.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What examination is requested?

- A An independent line adjustment
- C A completed diagnosis

- B A device-setting change
- D A portable chest examination

2. Who identifies attached equipment?

- A The exercise author
- C A visitor

- B Priya
- D An absent interpreter

3. What has not happened?

- A Arrival at the bedside
- C A preparation discussion

- B A line disconnection
- D A request for coordination

[Answers and explanations: page 93.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

portable radiography

Radiographic imaging performed with mobile equipment at the patient's location.

IN USE coordinate portable radiography

bedside examination

Examination performed where the patient is receiving bedside care.

IN USE prepare a bedside examination

mobile X-ray unit

Movable equipment used to acquire radiographs outside a fixed room.

IN USE position the mobile X-ray unit

detector placement

Positioning the image receptor for the intended examination.

IN USE coordinate detector placement

attached line

Tube or connection linked to the patient's care.

IN USE identify attached lines

monitoring equipment

Devices used to observe relevant physiological information.

IN USE protect monitoring equipment

oxygen delivery device

Equipment providing prescribed oxygen support.

IN USE identify the oxygen delivery device

bedside coordination

Joint planning of activity around the patient's immediate care.

IN USE request bedside coordination

equipment clearance

Available space for positioning and moving equipment.

IN USE check equipment clearance

infection precaution

Measure intended to reduce transmission of infection.

IN USE follow infection precautions

isolation status

Applicable infection-control designation for the patient or area.

IN USE verify isolation status

cleaning protocol

Approved process for decontaminating equipment or surfaces.

IN USE follow the cleaning protocol

Precision check

Recognizing an attached line does not authorize disconnecting or adjusting it. Identify the responsible professional and coordinate the actual preparation.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

patient support

Assistance needed to maintain a suitable and safe position.

IN USE arrange patient support

line management

Authorized handling and protection of clinical tubing or connections.

IN USE coordinate line management

team readiness

Confirmation that required people and preparations are in place.

IN USE confirm team readiness

equipment adjustment

Change to a device's position or settings within authorized practice.

IN USE request authorized equipment adjustment

handoff acknowledgment

Confirmation that a transfer of information has been received.

IN USE obtain handoff acknowledgment

bedside constraint

Limitation affecting an examination at the patient's location.

IN USE communicate bedside constraints

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Portable does not mean informal. Identity, examination, infection-control, movement, and exposure checks still follow the department process.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE REQUEST

"I am here for the requested portable chest examination."

NAME THE DEPENDENCY

"I need the appropriate team to coordinate movement first."

CLARIFY RESPONSIBILITY

"Who is authorized to manage this device?"

STATE A LIMITATION

"The available space limits where the unit can be positioned."

ASK ABOUT CONSTRAINTS

"What attached equipment affects positioning?"

PROTECT CONNECTIONS

"I will not disconnect that line or change its settings."

CHECK PREPARATION

"Please confirm when the bedside preparation is complete."

CLOSE THE HANDOFF

"Please acknowledge the preparation plan before we proceed."

Need before

Makes preparation a prerequisite rather than an optional courtesy.

Manage versus move

Clinical equipment may require authorized management, not merely physical relocation.

Please confirm when

Requests a clear readiness signal instead of assuming silence means agreement.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which request identifies a dependency?

- A** Silence means ready.
- B** I need authorized line management before movement.
- C** I will remove anything in the way.
- D** Portable imaging needs no checks.

2. What does portable describe?

- A** Equipment and examination location
- B** Reduced professional responsibility
- C** Permission to alter devices
- D** A diagnostic conclusion

3. Which action is unsupported?

- A** Coordinating detector placement
- B** Disconnecting an attached line independently
- C** Asking who manages the line
- D** Checking isolation status

4. What confirms readiness?

- A** The radiographer's arrival alone
- B** A clear floor alone
- C** Explicit acknowledgment from the relevant team
- D** An unanswered request

Answers and explanations: [page 93](#).

The bedside team stays connected

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

05

WORD + PHRASE BANK

handoff acknowledgment / patient support / oxygen delivery device / bedside coordination / isolation status / equipment clearance / team readiness / line management / cleaning protocol / detector placement

- 01 **Sam:** I am here for the requested portable chest examination. Before positioning, can we review the bedside constraints together?
- 02 **Priya:** Yes. There are attached lines and monitoring devices, and movement needs coordination with the appropriate bedside staff.
- 03 **Sam:** This ¹ _____ is important. I will not treat the connections as objects I can simply move aside.
- 04 **Priya:** Thank you. I can identify them and confirm who is authorized to manage each device.
- 05 **Sam:** Please include the ² _____ in that review. I will not disconnect it or change any setting.
- 06 **Priya:** Understood. We also need to confirm the precautions currently applying to this patient and room.
- 07 **Sam:** I will check the ³ _____ and follow the required preparation for the equipment and examination.
- 08 **Priya:** Space is limited on the far side of the bed. Does that affect where your unit can go?
- 09 **Sam:** Yes. We need sufficient ⁴ _____ without interfering with care or assuming that a device can be relocated.
- 10 **Priya:** I will ask the appropriate colleague to help coordinate the movement and protect the connections.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Sam:** That will support ⁵ _____. We should agree on the sequence before placing anything or moving the patient.
- 12 **Priya:** The patient also needs support to maintain the position. We have not yet assessed the exact assistance.
- 13 **Sam:** Then ⁶ _____ remains an outstanding preparation. I will not describe the bedside as ready yet.
- 14 **Priya:** I will confirm the assistance and line responsibilities through our actual clinical process.
- 15 **Sam:** Please name who will handle ⁷ _____ so that nobody assumes someone else has accepted that responsibility.
- 16 **Priya:** Agreed. I will give you an explicit update when the required people and preparations are in place.
- 17 **Sam:** That confirms ⁸ _____, rather than treating an unanswered request as permission to begin.
- 18 **Priya:** Afterward, the equipment also needs the appropriate cleaning before it leaves the area.
- 19 **Sam:** Yes. I will follow the ⁹ _____ and communicate any unresolved issue through the department process.
- 20 **Priya:** We have a clear ¹⁰ _____: preparation first, explicit readiness, then the examination under the actual safety process.

ANSWERS / EXPLANATIONS: PAGE 94

1. bedside coordination | 2. oxygen delivery device | 3. isolation status | 4. equipment clearance | 5. detector placement | 6. patient support | 7. line management | 8. team readiness | 9. cleaning protocol | 10. handoff acknowledgment

Readiness is not yet confirmed

F Say it

The nurse has requested transfer assistance, but the assisting colleague has not arrived. The radiographer waits for the actual readiness confirmation.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

acknowledgment / requested / arrived / confirmed

Nurse: "Transfer assistance has been ¹ _____."

Radiographer: "The colleague has not yet ² _____."

Nurse: "Readiness is not yet ³ _____."

Radiographer: "I will wait for the actual ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. requested | 2. arrived | 3. confirmed | 4. acknowledgment

Rehearse with a fixed script

1. Read the corrected bedside exchange, emphasizing the preparation dependencies.
2. Swap roles. Name the responsible team without assigning yourself an unauthorized equipment task.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A repeat is a reviewed decision

A Read the situation

Describe image quality precisely and distinguish technical review from diagnosis.

Radiographer Ari identifies motion affecting a chest image and asks senior radiographer Devon to review technical adequacy. The case does not supply a diagnostic interpretation or authorize an automatic repeat. The team will assess whether the image answers the clinical question and whether any additional acquisition is justified under the actual process.

PEOPLE / Ari: Radiographer | Devon: Senior radiographer

06

DESCRIBE THE LIMITATION, NOT THE BLAME

Use observable technical language. A patient may move because of pain, breathlessness, anxiety, or an unclear instruction; do not label the person difficult.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What limitation is identified?

- A Motion affecting the image
- C An incorrect diagnosis

- B A confirmed disease
- D A completed repeat

2. Who reviews technical adequacy?

- A An automated language answer
- C The patient's employer

- B Devon
- D A visitor

3. What is not authorized automatically?

- A A repeat exposure
- C Describing motion

- B A review request
- D Preserving the image

Answers and explanations: [page 95](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

motion artifact

Image distortion caused by movement during acquisition.

IN USE identify motion artifact

technical adequacy

Fitness of an image's technical features for its intended purpose.

IN USE review technical adequacy

diagnostic adequacy

Whether available images sufficiently address the clinical question.

IN USE assess diagnostic adequacy

repeat exposure

Additional radiation exposure to acquire another image.

IN USE justify a repeat exposure

collimation

Restriction of the X-ray beam to the intended area.

IN USE assess collimation

exposure index

System indicator related to detector exposure under specified conditions.

IN USE interpret the exposure index

detector exposure

Radiation reaching the image receptor during acquisition.

IN USE review detector exposure

image noise

Unwanted variation affecting the visual signal in an image.

IN USE assess image noise

image contrast

Difference in appearance between image regions or structures.

IN USE assess image contrast

spatial resolution

Ability to distinguish nearby structures in an image.

IN USE describe spatial resolution

anatomical coverage

Extent of the required anatomy included in an image.

IN USE check anatomical coverage

positioning error

Departure from the intended patient or anatomical position.

IN USE describe a positioning error

06

Precision check

A technical limitation is not a diagnosis. Exposure index is not a direct patient-dose reading, and a displayed image should not be judged from brightness alone.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

exposure factor

Technical parameter affecting an imaging exposure.

IN USE review exposure factors

image processing

Computational treatment of acquired image data.

IN USE distinguish image processing

reject analysis

Review of rejected images to identify patterns and improvement opportunities.

IN USE conduct reject analysis

quality assurance

Organized activities checking and improving process quality.

IN USE support quality assurance

technical review

Assessment of relevant acquisition and image-quality features.

IN USE request a technical review

additional acquisition

Further collection of image data after an initial acquisition.

IN USE assess additional acquisition

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Retain images and follow the actual review and repeat process. Do not hide a rejected image or automatically repeat solely because a picture looks imperfect.

Phrases you can actually use

C Notice the language / 1 of 2

DESCRIBE THE ISSUE

"There is visible motion affecting this image."

AVOID BLAME

"I will document the limitation without labeling the patient."

SEPARATE MEASURES

"The exposure index is not a direct patient-dose reading."

CLARIFY THE PURPOSE

"We need adequate information, not merely a prettier picture."

REQUEST REVIEW

"Could you review whether the image answers the clinical question?"

AVOID AUTOMATIC REPETITION

"A repeat needs the appropriate assessment and justification."

PRESERVE THE RECORD

"I will retain the image within the approved record."

SUPPORT IMPROVEMENT

"Let us identify what can improve the next instruction."

06

Affecting versus invalidating

A limitation may affect an image without automatically making the entire examination unusable.

Whether

Frames adequacy as a question for review rather than a conclusion already reached.

Additional versus replacement

An additional exposure has consequences; it is not merely an edit to an existing picture.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

06

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

06

1. Which statement is precise?

- A** Every imperfect image must be repeated.
- B** Motion proves disease.
- C** Motion affects this image and needs review.
- D** The patient ruined the study.

2. What is not a direct patient-dose reading?

- A** A verified dose report
- B** Exposure index
- C** A documented patient-dose quantity
- D** An appropriately measured dose metric

3. Why review before repeating?

- A** To assess adequacy and justify any additional acquisition
- B** To hide the first image
- C** To guarantee a normal diagnosis
- D** To make every image identical

4. Which practice preserves integrity?

- A** Retaining the image through the approved process
- B** Deleting evidence of motion
- C** Reusing an old image as new
- D** Changing the record invisibly

Answers and explanations: page 95.

A repeat is a reviewed decision

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

additional acquisition / motion artifact / exposure index / repeat exposure / technical adequacy / Quality assurance / diagnostic adequacy / technical review / Reject analysis / anatomical coverage

06

- 01 **Ari:** This chest image has visible motion. Could you review its technical adequacy before we decide whether anything further is needed?
- 02 **Devon:** Yes. Describe the limitation specifically, and keep the original image available in the approved record.
- 03 **Ari:** The ¹ _____ affects part of the image. I have not concluded that the whole examination is unusable.
- 04 **Devon:** Good. We need to assess the actual clinical question, not simply aim for a more attractive picture.
- 05 **Ari:** So ² _____ is different from making every image look technically perfect.
- 06 **Devon:** Exactly. The relevant professional review determines what information is sufficient and what limitation matters.
- 07 **Ari:** I also checked the ³ _____, rather than relying only on the overall brightness of the displayed image.
- 08 **Devon:** That is useful. Display appearance and processing do not independently establish appropriate exposure or adequacy.
- 09 **Ari:** The system shows an ⁴ _____. I should not describe that as a direct reading of the patient's dose.
- 10 **Devon:** Correct. Interpret the indicator according to the actual system and context, without substituting it for another quantity.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

06

- 11 **Ari:** Does identifying the motion authorize me to acquire another image immediately?
- 12 **Devon:** No. A ⁵ _____ needs appropriate assessment and justification under our process.
- 13 **Ari:** I will also avoid recording the patient as difficult. We have not established why the movement occurred.
- 14 **Devon:** Yes. ⁶ _____ should examine the process, including whether our instructions were clear and manageable.
- 15 **Ari:** If another acquisition is authorized, the instruction may need to be explained or rehearsed differently.
- 16 **Devon:** That belongs in the ⁷ _____, alongside the actual clinical and technical constraints.
- 17 **Ari:** The original image will remain available even if the process classifies it as unsuitable.
- 18 **Devon:** Correct. ⁸ _____ depends on an accurate record, not deleting evidence of a problem.
- 19 **Ari:** I will document the limitation and the review outcome before treating an ⁹ _____ as agreed.
- 20 **Devon:** That keeps ¹⁰ _____, diagnostic interpretation, and the decision about another exposure distinct.

ANSWERS / EXPLANATIONS: PAGE 96

1. motion artifact | 2. diagnostic adequacy | 3. anatomical coverage | 4. exposure index | 5. repeat exposure | 6. Quality assurance | 7. technical review | 8. Reject analysis | 9. additional acquisition | 10. technical adequacy

A missing area needs review

F Say it

A required anatomical area is not fully included. The image is preserved. The appropriate reviewer has not yet decided whether additional acquisition is needed.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

incomplete / decided / available / assessment

Radiographer: "The anatomical coverage is ¹ _____."

Reviewer: "The image must remain ² _____."

Radiographer: "Additional acquisition is not yet ³ _____."

Reviewer: "We need the appropriate ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. incomplete | 2. available | 3. decided | 4. assessment

Rehearse with a fixed script

1. Read the corrected exchange. Distinguish the limitation from the decision about repeating.
2. Swap roles. Explain exposure index without giving a dose estimate or selecting exposure settings.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Explain the process without interpreting the image

A Read the situation

Set a helpful professional boundary and give a concrete route for results.

After an examination, patient Jules asks radiographer Ren whether a visible mark means cancer. Ren has not issued a diagnostic interpretation. The reporting clinician will interpret the images, and the referring service will explain the result through its actual process. No reporting deadline is supplied. Ren checks how Jules can contact the service rather than promising a result time.

PEOPLE / Ren: Radiographer | Jules: Patient

07

A BOUNDARY SHOULD INCLUDE A NEXT STEP

A bare I cannot say may feel dismissive. Acknowledge the concern, explain your role, and identify the person or service responsible for interpretation and communication.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Jules ask about?

- A** A confirmed cancer diagnosis
- C** A guaranteed deadline

- B** A signed report
- D** Whether a mark means cancer

2. Who interprets the images?

- A** The language learner independently
- C** The appointment system

- B** An employer
- D** The reporting clinician

3. What is not supplied?

- A** Jules's concern
- C** A completed examination

- B** A referring service
- D** A reporting deadline

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

image interpretation

Professional assessment of images for clinical meaning.

IN USE arrange image interpretation

reporting clinician

Authorized professional responsible for the imaging interpretation.

IN USE identify the reporting clinician

radiology report

Recorded interpretation of an imaging examination.

IN USE review the radiology report

preliminary report

Initial interpretation that may be updated after further review.

IN USE distinguish a preliminary report

final report

Completed authorized report under the relevant reporting process.

IN USE communicate the final report

incidental finding

Finding outside the main reason for the examination.

IN USE explain an incidental finding

comparison images

Earlier images used to assess relevant differences.

IN USE obtain comparison images

referring service

Clinical service that requested or coordinates the examination.

IN USE contact the referring service

result communication

Process of explaining a result to the appropriate recipient.

IN USE confirm result communication

report turnaround

Time between the relevant examination stage and report availability.

IN USE verify report turnaround

critical result

Finding requiring urgent communication through the relevant clinical pathway.

IN USE escalate a critical result

report addendum

Additional or corrected information appended to a report.

IN USE review a report addendum

Precision check

A visible feature is not an established diagnosis. Acquiring the image does not automatically authorize diagnostic interpretation.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

clinical correlation

Interpretation of a finding alongside other clinical information.

IN USE explain clinical correlation

patient portal

Secure system allowing a patient to access relevant health information.

IN USE explain patient-portal access

result inquiry

Request for information about an examination result.

IN USE handle a result inquiry

scope of practice

Duties a professional is trained and authorized to perform.

IN USE respect scope of practice

follow-up contact

Named person or service for subsequent communication.

IN USE provide a follow-up contact

unverified impression

Initial belief not established by the appropriate interpretation.

IN USE avoid an unverified impression

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Report availability and explanation to the patient are separate steps. Do not promise a deadline or assume portal release completes clinical communication.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE CONCERN

"I can hear that the mark is worrying you."

NAME THE INTERPRETER

"The reporting clinician will interpret the examination."

GIVE A ROUTE

"Let us confirm how the referring service will explain the result."

SEPARATE ACCESS AND EXPLANATION

"Seeing a report online is not the same as having it explained."

CLARIFY YOUR ROLE

"My role here is to acquire the images."

AVOID SPECULATION

"I cannot identify that as a diagnosis from this conversation."

LIMIT TIMING

"I need to verify the reporting arrangements rather than guess."

OFFER A CONCRETE CONTACT

"I can help identify the appropriate contact for your question."

07

Will interpret versus has confirmed

Distinguishes a pending professional process from a completed finding.

Cannot identify as

Sets a precise boundary without dismissing the person's concern.

Verify rather than guess

Avoids turning an uncertain administrative timeframe into a promise.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response is most helpful?

- A** I cannot say; goodbye.
- B** It is definitely harmless.
- C** It must be cancer.
- D** I cannot interpret that, but I can confirm the reporting route.

2. Which distinction matters?

- A** Arrival and report completion as identical
- B** Report access and clinical explanation
- C** The screen color and the diagnosis
- D** A mark and a confirmed cancer as identical

3. What timing claim is unsupported?

- A** No deadline is supplied here.
- B** The service can clarify its process.
- C** The result will definitely be ready tomorrow.
- D** I will check the arrangements.

4. What does clinical correlation mean?

- A** Guaranteeing a benign finding
- B** Interpreting a finding with other clinical information
- C** Matching the report's colors
- D** Deleting an uncertain image

Answers and explanations: [page 97](#).

Explain the process without interpreting the image

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Comparison images / reporting clinician / Result communication / report turnaround / follow-up contact / unverified impression / patient portal / result inquiry / Clinical correlation / scope of practice

- 01 **Jules:** I saw a mark on the screen. Does that mean cancer, or can you tell me it is harmless?
- 02 **Ren:** I can hear that it worries you. My role here is to acquire images, not give their diagnostic interpretation.
- 03 **Jules:** But you see these pictures every day. Surely you have an idea of what that mark means.
- 04 **Ren:** An ¹ _____ would not give you a reliable answer. I do not want to mislead you in either direction.
- 05 **Jules:** Then who can answer the question properly, and how will the information get back to me?
- 06 **Ren:** The ² _____ interprets the examination, and we should confirm how your referring service communicates the result.
- 07 **Jules:** Is a report the same thing as someone discussing what it means for my situation?
- 08 **Ren:** No. ³ _____ includes explaining it in context, not merely creating a document in the system.
- 09 **Jules:** I sometimes see reports on my phone before anyone has spoken to me about them.
- 10 **Ren:** A ⁴ _____ can provide access, but seeing the words is not the same as discussing their meaning with the appropriate clinician.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Jules:** If it says correlate clinically, does that mean the picture did not work?
- 12 **Ren:** ⁵ _____ means considering the finding with other clinical information. It is not automatically a statement that acquisition failed.
- 13 **Jules:** Could an earlier examination help the person who is preparing the report?
- 14 **Ren:** ⁶ _____ may provide useful context. The team follows its process for obtaining and reviewing them.
- 15 **Jules:** Can you promise that someone will call tomorrow? Waiting without a contact is the hardest part.
- 16 **Ren:** I need to verify the ⁷ _____ arrangements rather than guess. No definite reporting time has been established in this conversation.
- 17 **Jules:** I understand the limit, but I would still appreciate knowing whom to contact with the question.
- 18 **Ren:** Let us identify the ⁸ _____ and the referring service's route for discussing your result.
- 19 **Jules:** That is more useful than a guess. I can ask about the report through that route.
- 20 **Ren:** Yes. Respecting ⁹ _____ still allows a helpful handoff for your ¹⁰ _____.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. unverified impression | 2. reporting clinician | 3. Result communication | 4. patient portal | 5. Clinical correlation | 6. Comparison images | 7. report turnaround | 8. follow-up contact | 9. scope of practice | 10. result inquiry

An online report has appeared

F Say it

Pat can see a report online but has not discussed it with the referring clinician. No interpretation or follow-up decision is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

explanation / contact / here / portal

Pat: "The report is visible in the ¹ _____."

Radiographer: "Access is not the same as clinical ² _____."

Pat: "I need the appropriate ³ _____."

Radiographer: "No follow-up decision is supplied ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. portal | 2. explanation | 3. contact | 4. here

Rehearse with a fixed script

1. Read the corrected result discussion, pairing every boundary with a useful next step.
2. Swap roles. Respond to the request for certainty without supplying a diagnosis or a deadline.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Correct the record without hiding the history

A Read the situation

Describe a discrepancy factually and request an authorized correction with an audit trail.

Radiographer Kim notices that an image label may not match the verified examination details. Kim stops further release and contacts supervisor Noor through the department process. The original record is preserved. The team has not established how the mismatch arose or whether any incorrect information reached another service. This is a communication case, not an instruction for editing clinical images.

PEOPLE / Kim: Radiographer | Noor: Supervisor

08

A CORRECTION SHOULD REMAIN TRACEABLE

Report what is known promptly, identify what remains unverified, and preserve the record. Do not turn a suspected mismatch into a proven cause or quietly erase its history.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Kim notice?

- A** A normal diagnostic report
- C** A proven cause

- B** A possible label mismatch
- D** A confirmed external disclosure

2. What happens to the original record?

- A** It is replaced without history
- C** It is preserved

- B** It is reused for another patient
- D** It is secretly deleted

3. What remains unestablished?

- A** How the mismatch arose
- C** That release was paused

- B** That Kim contacted Noor
- D** That a concern exists

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

label discrepancy

Conflict between an image label and verified examination information.

IN USE report a label discrepancy

near miss

Event caught before the relevant potential harm occurs.

IN USE report a potential near miss

incident report

Formal record of an event through the applicable reporting system.

IN USE submit an incident report

audit trail

Traceable history of actions and changes in a record.

IN USE preserve the audit trail

record integrity

Accuracy and trustworthiness of a maintained record.

IN USE protect record integrity

data correction

Authorized amendment of inaccurate recorded information.

IN USE request a data correction

release hold

Temporary prevention of further distribution pending review.

IN USE apply an authorized release hold

downstream recipient

Person or system receiving information later in the workflow.

IN USE identify downstream recipients

root cause

Underlying factor established through appropriate investigation.

IN USE investigate the root cause

contributing factor

Circumstance that may have influenced an event.

IN USE identify contributing factors

event chronology

Ordered account of what happened and when.

IN USE document the event chronology

verified detail

Information checked against an appropriate source.

IN USE distinguish verified details

Precision check

A possible mismatch is not an established root cause. A near miss can be confirmed only when the relevant facts support that classification.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

unconfirmed impact

Possible consequence not yet established by review.

IN USE state unconfirmed impact

notification route

Approved path for informing the relevant people.

IN USE follow the notification route

correction authorization

Approval required to amend a relevant record.

IN USE obtain correction authorization

preserved original

Original information retained through the approved process.

IN USE maintain the preserved original

system reconciliation

Process of aligning related records across relevant systems.

IN USE coordinate system reconciliation

learning review

Structured review intended to improve future practice.

IN USE support a learning review

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

08

Precision check

Follow the real incident, correction, and notification processes. Never conceal evidence, overwrite history informally, or assume that fixing one display corrects every linked system.

Phrases you can actually use

C Notice the language / 1 of 2

LEAD WITH THE DISCREPANCY

"The label may not match the verified examination details."

PRESERVE UNCERTAINTY

"The cause and downstream impact are not yet established."

REQUEST AUTHORIZATION

"Please confirm the approved correction route."

CHECK LINKED RECORDS

"We need to verify which systems and recipients are affected."

STATE THE CONTAINMENT

"Further release is paused through our process."

KEEP THE ORIGINAL

"I have preserved the original record."

SEPARATE ACTION AND INVESTIGATION

"The immediate containment does not settle the root cause."

CLOSE FACTUALLY

"I will record the confirmed actions and outstanding questions."

08

May not match

Describes a suspected discrepancy without pretending the investigation is complete.

Not yet established

Preserves uncertainty about cause and impact without minimizing the event.

Corrected versus concealed

A traceable authorized amendment differs from an invisible alteration.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening is factual?

- A** I erased it, so it no longer matters.
- B** The label may not match verified details.
- C** I know who caused this without review.
- D** Nothing happened anywhere.

2. What must remain traceable?

- A** The original record and authorized changes
- B** Only the final screenshot
- C** A guessed explanation
- D** An informal memory

3. Which conclusion is premature?

- A** The supervisor was contacted.
- B** The original is preserved.
- C** No downstream recipient was affected.
- D** Impact is not established.

4. What is system reconciliation for?

- A** Replacing investigation with a guess
- B** Automatically changing every record
- C** Aligning related records across affected systems
- D** Hiding the first version

Answers and explanations: page 99.

Correct the record without hiding the history

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

downstream recipient / event chronology / audit trail / preserved original / release hold / record integrity / Unconfirmed impact / correction authorization / label discrepancy / root cause

08

- 01 **Kim:** I have noticed a possible mismatch between the image label and the verified examination details. Further release is paused.
- 02 **Noor:** Thank you for escalating it. Preserve the original record and state exactly what you have verified so far.
- 03 **Kim:** The ¹ _____ is the concern. I have not established how it arose or whether another service received it.
- 04 **Noor:** Good. Keep the event description separate from any assumption about who or what caused it.
- 05 **Kim:** I have applied the authorized ² _____ through our process, rather than making an undocumented change to the image.
- 06 **Noor:** We need the approved correction route and an account of what happened in order.
- 07 **Kim:** I will give an ³ _____ using verified times and actions, marking anything I have not confirmed.
- 08 **Noor:** Include the point at which you noticed the issue and the people contacted afterward.
- 09 **Kim:** The ⁴ _____ remains available. I have not deleted it or substituted an earlier image.
- 10 **Noor:** That protects the record while the appropriate people determine the correction and notification requirements.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Kim:** Should I call this a near miss now, or wait until the impact is known?
- 12 **Noor:** Use the appropriate classification after review. ⁵ _____ is not the same as proof that no one received incorrect information.
- 13 **Kim:** We need to identify every relevant ⁶ _____ through the authorized investigation rather than assume the local screen is the only record.
- 14 **Noor:** Correct. Fixing one display does not prove that all related records are aligned.
- 15 **Kim:** I will request ⁷ _____ and keep the actions visible in the approved system.
- 16 **Noor:** We also need the ⁸ _____, including what was changed, by whom, and through which process.
- 17 **Kim:** Once the immediate issue is contained, the investigation can examine contributing factors without guessing at blame.
- 18 **Noor:** Yes. Establishing a ⁹ _____ requires investigation; it is not settled by the first report.
- 19 **Kim:** I will complete the required report and flag the unresolved system and recipient questions.
- 20 **Noor:** That supports ¹⁰ _____ and a useful learning review while the actual correction process continues.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. label discrepancy | 2. release hold | 3. event chronology | 4. preserved original | 5. Unconfirmed impact | 6. downstream recipient | 7. correction authorization | 8. audit trail | 9. root cause | 10. record integrity

One linked system is still unchecked

F Say it

An authorized correction is complete in the local record. A linked system has not yet been checked. The original and change history are retained.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unchecked / retained / confirmed / complete

08

Reporter: "The local correction is ¹ _____."

Supervisor: "The linked system remains ² _____."

Reporter: "The original and history are ³ _____."

Supervisor: "Full reconciliation is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. complete | 2. unchecked | 3. retained | 4. confirmed

Rehearse with a fixed script

1. Read the corrected incident exchange, separating verified actions from unresolved cause and impact.
2. Swap roles. Practice requesting a traceable correction without specifying an unauthorized editing method.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Positioning instructions through an interpreter

E More workplace conversations / 1-10

Coordinate interpreted movement instructions without relying on a companion to guess the meaning.

Radiographer Ari and qualified interpreter Sol prepare an examination for patient Hana. The script shows coordination before speaking with Hana. The actual examination and patient ability determine the instructions. No positioning technique or permission to proceed is supplied by this language case.

PEOPLE / Ari: Radiographer | Sol: Qualified interpreter

09

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

single-step instruction / understanding check / informed agreement / pause signal / patient response / role boundaries

- 01 **Ari:** I want Hana to understand each instruction before moving. Let us agree on short segments and a clear pause.
- 02 **Sol:** Please speak directly to Hana and let me interpret the complete instruction before adding another movement.
- 03 **Ari:** I will use a ¹ _____ rather than combine turning, lifting, and breathing into one long sentence.
- 04 **Sol:** That will help. If a term is unclear, I will request clarification rather than choose a movement by guesswork.
- 05 **Ari:** I also need to check what Hana can comfortably do before describing any position as agreed.
- 06 **Sol:** I can interpret the question and response without replacing the patient's account with my own assessment.
- 07 **Ari:** We should preserve the ² _____ exactly, especially if an instruction causes pain or confusion.
- 08 **Sol:** Yes. An interpreter's fluent answer must not be mistaken for the patient's agreement or ability to move.
- 09 **Ari:** Hana's companion has offered to show the movement, but the actual ability and safe approach still need assessment.
- 10 **Sol:** Qualified interpretation supports communication; it does not authorize a companion to move the patient or make clinical decisions.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Ari:** I will ask Hana to explain the instruction back before any actual acquisition, when appropriate.
- 12 **Sol:** That ³ _____ should be directed to Hana, not to me as though I were the patient.
- 13 **Ari:** If the instruction needs revision, I will say so clearly rather than change it halfway through your interpretation.
- 14 **Sol:** Please do. A transparent correction prevents the patient from receiving two incompatible instructions without knowing which is current.
- 15 **Ari:** We also need an agreed way for Hana to request a pause during the preparation.
- 16 **Sol:** A ⁴ _____ can be explained and checked before the procedure begins.
- 17 **Ari:** I will not assume that a nod means every safety question has been answered.
- 18 **Sol:** Nor should silence be treated as ⁵ _____ without the actual required process.
- 19 **Ari:** Once the explanation is clear, we will proceed only under the real examination checks and authorized plan.
- 20 **Sol:** That preserves ⁶ _____ while making the instructions accessible to Hana.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. single-step instruction | 2. patient response | 3. understanding check | 4. pause signal | 5. informed agreement | 6. role boundaries

Two movements were combined

F Say it in a new situation

The radiographer gives two movement instructions together. The interpreter requests separate instructions before the patient moves. The patient has not yet agreed to the movement.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

agreed / movement / proceeding / instructions

09

Interpreter: "Please separate the two ¹ _____."

Radiographer: "I will give one ² _____ at a time."

Interpreter: "The patient has not yet ³ _____."

Radiographer: "We will check understanding before ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. instructions | 2. movement | 3. agreed | 4. proceeding

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

The imaging worklist is unavailable

E More workplace conversations / 1-10

Use the approved downtime process without creating duplicate or guessed patient records.

The imaging worklist is unavailable. Radiographer Ren contacts lead technologist Dev. They use the approved downtime process for identity, request verification, acquisition records, and later reconciliation. No substitute identifiers, device operation instructions, or authorization to bypass checks are supplied.

PEOPLE / Ren: Radiographer | Dev: Lead technologist

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

acquisition record / downtime procedure / patient matching / reconciliation ownership / Duplicate prevention / system outage

10

- 01 **Ren:** The worklist is unavailable. I can see an appointment message, but I do not want to treat it as a verified order.
- 02 **Dev:** Use the approved downtime process. The outage does not remove identity or examination-verification requirements.
- 03 **Ren:** We need to confirm whether this is a ¹ _____ affecting the worklist rather than assume every system is unavailable.
- 04 **Dev:** Correct. State what is affected and what remains accessible without guessing about the technical cause.
- 05 **Ren:** I will not create a new record from a remembered name or use another patient's examination as a placeholder.
- 06 **Dev:** That protects ² _____ and avoids introducing a second identity problem while addressing the first.
- 07 **Ren:** What should we use for recording the examination if the normal electronic workflow cannot be completed?
- 08 **Dev:** Follow the actual ³ _____, including the authorized documentation and verification steps for this service.
- 09 **Ren:** We also need a clear record of what was acquired and when, so the information can be reconciled later.
- 10 **Dev:** Yes. Preserve the required ⁴ _____ under the approved process rather than relying on memory after the system returns.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Ren:** Could entering the same request again create a duplicate if the original reappears?
- 12 **Dev:** It could create a discrepancy. Do not improvise another order or identifier outside the authorized process.
- 13 **Ren:** I will tell the team which examinations remain incomplete in the electronic workflow.
- 14 **Dev:** A clear status prevents an unfinished electronic step from being mistaken for an examination that never occurred.
- 15 **Ren:** When the system is restored, someone needs to check that the records match rather than assume the outage fixed itself.
- 16 **Dev:** Assign ⁵ _____ and follow the actual review process.
- 17 **Ren:** That review should also check for duplicate entries and any information still missing.
- 18 **Dev:** Exactly. ⁶ _____ is part of maintaining a trustworthy record.
- 19 **Ren:** I will report the affected workflow, use the approved process, and keep unresolved items visible.
- 20 **Dev:** Good. Restoration of system access is not proof that every examination record has been correctly reconciled.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. system outage | 2. patient matching | 3. downtime procedure | 4. acquisition record | 5. reconciliation ownership | 6. Duplicate prevention

Access is restored, reconciliation is not complete

F Say it in a new situation

The worklist becomes available again. Two downtime records still need verification against the restored system. No duplicates have been ruled out yet.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

complete / restored / unchecked / verification

10

Technologist: "System access is ¹ _____."

Lead: "Two records still need ² _____."

Technologist: "Reconciliation is not ³ _____."

Lead: "Possible duplicates remain ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 102

1. restored | 2. verification | 3. complete | 4. unchecked

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A patient asks to stop the examination

E More workplace conversations / 1-10

Respond to a request to stop without interpreting distress as permission to continue.

During preparation, patient Jo asks radiographer Kim to stop because the position is painful and a previous experience was distressing. Kim stops the preparation and seeks the appropriate clinical review. No emergency exception, restraint authorization, or alternative examination decision is supplied.

PEOPLE / Jo: Patient | Kim: Radiographer

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

request to stop / physical contact / clinical review / positioning intolerance / revised agreement / support arrangement

11

- 01 **Jo:** Please stop. That position hurts, and I am remembering a previous examination that was very distressing.
- 02 **Kim:** I am stopping the preparation. Thank you for telling me; we need to understand what is difficult before proceeding.
- 03 **Jo:** I was afraid that once I entered the room I would have no say about what happened next.
- 04 **Kim:** Entering the room is not unlimited permission. Your ¹ _____ needs to be heard and handled through the actual clinical process.
- 05 **Jo:** I still want help with the problem that brought me here, but I cannot tolerate this position now.
- 06 **Kim:** That distinction matters. A ² _____ is not automatically refusal of all care.
- 07 **Jo:** Could you explain what you are going to do before touching my arm again?
- 08 **Kim:** Yes. I will explain and seek the appropriate agreement before any proposed ³ _____ rather than surprise you with another movement.
- 09 **Jo:** I do not want to describe every detail of the previous experience in order to be believed.
- 10 **Kim:** You do not need to disclose unnecessary personal detail to have the current pain and request acknowledged.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Jo:** What happens if the original position is not possible for me today?
- 12 **Kim:** We need the appropriate ⁴ _____ and authorized approach, not an improvised alternative that I assume will be suitable.
- 13 **Jo:** Will the record say I was difficult, or will it explain what I actually said?
- 14 **Kim:** I will document the pain, the position, and your request factually, avoiding a judgment about your character.
- 15 **Jo:** I would like a support person involved if that can be arranged appropriately.
- 16 **Kim:** We can discuss the actual ⁵ _____ and the relevant safety and privacy requirements.
- 17 **Jo:** I appreciate knowing that stopping does not mean I will be abandoned without an explanation.
- 18 **Kim:** We should provide a clear next step and contact, while keeping the decision about the examination with the appropriate clinical process.
- 19 **Jo:** Then we can pause, review the options, and avoid pretending that I already agreed to another attempt.
- 20 **Kim:** Correct. A ⁶ _____ would need the actual discussion; it has not been assumed here.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. request to stop | 2. positioning intolerance | 3. physical contact | 4. clinical review | 5. support arrangement | 6. revised agreement

The patient agrees to discussion, not movement

F Say it in a new situation

A patient agrees to discuss an alternative approach after a pause. The patient has not agreed to another positioning attempt. The clinical review is pending.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

agreed / pending / discussion / step

Patient: "I agree to a ¹ _____."

Radiographer: "Another movement is not yet ² _____."

Patient: "The review remains ³ _____."

Radiographer: "We will explain the proposed next ⁴ _____."

11

ANSWERS / EXPLANATIONS: PAGE 103

1. discussion | 2. agreed | 3. pending | 4. step

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Resolving a request discrepancy

A Briefing checks / page 4

1. Which side appears on the request?

B. Left

The written request names the left wrist in the supplied case.

2. What does Morgan describe?

B. A right-wrist injury

Morgan describes the right wrist, which conflicts with the request.

3. What happens before acquisition?

C. Verification through the department process

The discrepancy must be resolved through the authorized process before acquisition.

D Language checks / page 9

1. Which question avoids leading?

D. Please tell me which wrist was injured.

The open factual question does not suggest the desired side to the patient.

2. What does an accession number identify?

C. A particular examination

The accession number identifies the examination record, not every patient or clinical detail.

3. Which action exceeds the supplied authorization?

A. Silently changing the request

The case does not authorize an undocumented independent change to the requested examination.

4. Which wording preserves both sources?

D. The request says left; the patient reports right.

Naming both sources preserves the conflict until the appropriate verification resolves it.

01 Conversation answers

Resolving a request discrepancy

E Dialogue gaps

Pages 10-11

1. imaging request. Turn 3. The imaging request is the authorized examination instruction and currently conflicts with the patient's reported side.

2. laterality. Turn 5. Laterality means the specified side and must be verified through the appropriate process rather than silently changed.

3. verification pause. Turn 7. A verification pause is a temporary stop to resolve conflicting information before the examination proceeds.

4. requesting clinician. Turn 9. The requesting clinician initiated the examination and is an appropriate contact for clarifying its intended scope.

5. accession number. Turn 11. An accession number identifies the examination record but cannot independently resolve the clinical side discrepancy.

6. patient identifier. Turn 13. A patient identifier is an approved identifying detail used to verify the person rather than infer identity from the appointment alone.

7. clinical indication. Turn 15. The clinical indication is the reason or question for imaging and helps the authorized team clarify the intended examination.

8. request amendment. Turn 17. A request amendment is a documented authorized change, not an informal substitution made without the required process.

9. Acquisition. Turn 19. Acquisition is obtaining the images and is paused here until the relevant discrepancy has been appropriately resolved.

10. resolved discrepancy. Turn 20. A resolved discrepancy means the conflict has been appropriately clarified and documented before the intended examination proceeds.

F Transfer conversation

Page 12

1. left

Jo: "My injured ankle is the left one."

Jo reports a left-ankle injury in the supplied scenario.

2. omits

Radiographer: "The request currently omits the side."

The missing laterality is the discrepancy requiring clarification before acquisition.

3. clarify

Jo: "The team will clarify the request."

The authorized team is being contacted to clarify the incomplete request.

4. acquired

Radiographer: "No images have been acquired."

The scenario explicitly states that acquisition has not yet begun.



02 Briefing and language checks

Adapting positioning instructions

A Briefing checks / page 13

1. What is difficult?

A. Raising the left shoulder

The case identifies one painful shoulder movement, not a general refusal.

2. What does Jamie do first?

D. Stops the attempted movement

Jamie stops the movement and checks the reported limitation.

3. What is not supplied?

D. An authorized alternative projection

The exercise does not prescribe the technical alternative for the examination.

D Language checks / page 18

1. Which record is most accurate?

A. Casey reports pain when raising the left shoulder.

The precise report describes the supplied movement limitation without inventing intent or examination findings.

2. Which instruction respects the limitation?

D. Stop there; do not force the movement.

Stopping acknowledges the reported difficulty while the appropriate approach is reviewed.

3. What should precede hands-on help?

D. Appropriate agreement and assessment

Assistance requires appropriate agreement and competence, not an unexpected physical intervention.

4. What is a projection?

B. A view defined by beam direction

Projection names an imaging view and does not itself authorize a technical change.

02 Conversation answers

Adapting positioning instructions

E Dialogue gaps

Pages 19-20

- 1. pain-limited movement.** Turn 3. Pain-limited movement accurately records the reported reason for stopping rather than assuming refusal or poor cooperation.
- 2. mobility limitation.** Turn 5. A mobility limitation describes restricted movement without assigning the patient a negative motive or a blanket refusal.
- 3. movement cue.** Turn 7. A movement cue directs one specific action and allows the patient's response to be checked before another instruction.
- 4. projection.** Turn 9. A projection is an imaging view defined by beam direction and its intended purpose should be distinguished from an improvised alternative.
- 5. support aid.** Turn 11. A support aid helps maintain a position but must be appropriate to the actual examination and the person's needs.
- 6. Physical assistance.** Turn 13. Physical assistance is hands-on support and requires appropriate agreement, assessment, and competence rather than surprise movement.
- 7. range of motion.** Turn 15. Range of motion describes the movement available at the joint and the reported limitation must remain part of the positioning discussion.
- 8. adapted technique.** Turn 17. An adapted technique is an appropriate modification based on the actual clinical and technical situation, not an automatic substitution.
- 9. breath-hold instruction.** Turn 19. A breath-hold instruction can be rehearsed when appropriate so the patient understands the sequence before acquisition.
- 10. positioning.** Turn 20. Positioning arranges the body for the intended image, and the instructions should respect the person's actual movement ability.

F Transfer conversation

Page 21

1. difficult

Drew: "Standing from this chair is difficult."

Drew reports a specific difficulty with standing from the chair.

2. assessment

Radiographer: "The transfer needs assessment."

The assistance required has not yet been assessed in this case.

3. refused

Drew: "I have not refused the examination."

A movement limitation is not equivalent to refusing imaging.

4. assistance

Radiographer: "I will request appropriate assistance."

The scenario calls for help from the appropriate team rather than an improvised transfer.

03 Briefing and language checks

Asking sensitive screening questions

A Briefing checks / page 22

1. What does Rowan request?

D. Privacy from a companion

Rowan specifically asks to answer the screening question privately.

2. What answer is supplied?

A. Uncertainty

Rowan is unsure, and that uncertainty must remain visible.

3. What decision is not supplied?

A. Whether to perform or cancel the examination

The language case does not determine the clinical examination decision.

D Language checks / page 27

1. Which entry matches Rowan's response?

A. Unsure

Uncertainty is the supplied answer and must not be silently converted into a negative.

2. Which statement overpromises?

B. There is absolutely no risk in every case.

A universal zero-risk statement is not supported by the individualized assessment.

3. What should guide the next step?

B. The actual protocol and clinical review

The relevant examination and clinical circumstances require the appropriate professional process.

4. Which phrase supports privacy?

B. We can discuss this without your companion present.

Offering a private conversation respects the patient's stated preference and sensitive information.

03 Conversation answers

Asking sensitive screening questions

03

E Dialogue gaps

Pages 28-29

1. private conversation. Turn 3. A private conversation respects the request to answer without the companion.

2. Screening uncertainty. Turn 5. Screening uncertainty must remain unresolved rather than being recorded as no.

3. examination protocol. Turn 7. The examination protocol governs this procedure, not every imaging situation universally.

4. benefit-risk discussion. Turn 9. A benefit-risk discussion weighs this examination's expected benefit and possible harm.

5. ionizing radiation. Turn 11. Ionizing radiation describes the radiation type, not a personal risk estimate.

6. screening response. Turn 13. The screening response records what the patient actually answered, including uncertainty, so the review is based on accurate information.

7. Justification. Turn 15. Justification considers whether the expected benefit warrants the examination in the actual clinical circumstances.

8. privacy preference. Turn 17. A privacy preference identifies the patient's wishes about who hears personal information and should be handled within the applicable rules.

9. radiologist review. Turn 19. Radiologist review is assessment by the imaging physician when required by the relevant process, not a decision supplied by this language exercise.

10. review outcome. Turn 20. The review outcome is the decision or findings after assessment and must not be presented as settled before that review occurs.

F Transfer conversation

Page 30

1. answer

Companion: "Can you share the screening answer?"

The companion asks for the content of the screening response.

2. authorized

Radiographer: "Sharing has not been authorized."

Rowan has not authorized disclosure of the private answer.

3. pending

Companion: "The review is still pending?"

The clinical review has not yet reached an outcome.

4. privacy

Radiographer: "I need to respect the requested privacy."

The scenario explicitly requires respecting Rowan's request for privacy.

04 Briefing and language checks

Helping a child cooperate

04

A Briefing checks / page 31

1. What worries Mina?

A. Whether the machine will touch her

Mina asks about the equipment and whether it will touch her.

2. What happens before acquisition?

C. Explanation and rehearsal

Lee explains the equipment and rehearses the instruction before acquisition.

3. What controls caregiver assistance?

D. The department safety process

Presence or assistance during exposure follows the actual safety process.

D Language checks / page 36

1. Which choice is genuine here?

B. Explanation first or demonstration first

The available choice concerns preparation order, not technical safety or diagnostic outcomes.

2. Which reassurance is honest?

B. I will explain before we change position.

Explaining the sequence is a commitment within the speaker's control rather than an unsupported guarantee.

3. What does a practice run do?

D. Rehearses an instruction before acquisition

Rehearsal supports understanding but does not settle consent, diagnosis, or every response.

4. Which terms require distinction?

C. Assent and caregiver permission

Child assent and legally required caregiver permission serve different roles in the actual setting.

04 Conversation answers

Helping a child cooperate

E Dialogue gaps

Pages 37-38

1. sensory description. Turn 3. A sensory description explains what the child may see, hear, or feel rather than relying on technical labels alone.

2. bounded choice. Turn 5. A bounded choice offers a real limited option, here the order of explanation and demonstration, within the available safe process.

3. age-appropriate explanation. Turn 7. An age-appropriate explanation adapts the language to the child's understanding without withholding necessary information or speaking only to adults.

4. practice run. Turn 9. A practice run rehearses the instruction before the actual image is acquired so uncertainty can be addressed beforehand.

5. exposure signal. Turn 11. The exposure signal is the cue associated with the actual acquisition and should be distinguished from a rehearsal.

6. protective arrangement. Turn 13. A protective arrangement is the approved safety setup and determines where another person can appropriately be during the examination.

7. preparation sequence. Turn 15. A preparation sequence tells the child what comes next and reduces surprise without making promises about every sensation or outcome.

8. stop signal. Turn 17. A stop signal gives the child a clear way to communicate a need for a pause and should be explained within the actual procedure.

9. assent. Turn 19. Assent is the child's affirmative agreement appropriate to the circumstances and does not replace any legally required caregiver permission.

10. demonstration. Turn 20. A demonstration shows the action before the child is asked to perform it and makes the instruction more concrete.

F Transfer conversation

Page 39

1. instruction

Noah: "I forgot the three-part instruction."

Noah reports difficulty remembering the instruction during the practice.

2. step

Radiographer: "We can use one step at a time."

The supplied adaptation is a single step at a time.

3. practice

Noah: "This is still practice."

The scenario states that acquisition has not yet begun.

4. taken

Radiographer: "No image has been taken."

The rehearsal occurs before the actual image is acquired.

05 Briefing and language checks

Coordinating portable imaging

A Briefing checks / page 40

1. What examination is requested?

D. A portable chest examination

The brief specifies a portable chest examination at the bedside.

2. Who identifies attached equipment?

B. Priya

Priya is the bedside nurse identifying lines and monitoring equipment.

3. What has not happened?

B. A line disconnection

The case explicitly states that no line has been disconnected.

D Language checks / page 45

1. Which request identifies a dependency?

B. I need authorized line management before movement.

The statement names the required preparation and does not assume authority to perform it.

2. What does portable describe?

A. Equipment and examination location

Portable describes how imaging reaches the patient, not a waiver of safeguards.

3. Which action is unsupported?

B. Disconnecting an attached line independently

No independent disconnection is authorized in the supplied case.

4. What confirms readiness?

C. Explicit acknowledgment from the relevant team

Readiness requires the actual preparations and team acknowledgment, not a single convenient cue.

05 Conversation answers

Coordinating portable imaging

E Dialogue gaps

Pages 46-47

- 1. bedside coordination.** Turn 3. Bedside coordination prepares movement and equipment management with the relevant staff.
- 2. oxygen delivery device.** Turn 5. An oxygen delivery device requires appropriately authorized handling; it is not furniture.
- 3. isolation status.** Turn 7. Isolation status identifies the actual infection-control requirements, not an assumed precaution.
- 4. equipment clearance.** Turn 9. Equipment clearance means the space required for appropriate positioning and movement.
- 5. detector placement.** Turn 11. Detector placement may require coordinated patient support and protection of attached equipment.
- 6. patient support.** Turn 13. Patient support is the assistance required for the position and must be assessed rather than assumed from the imaging request.
- 7. line management.** Turn 15. Line management involves authorized protection and handling of clinical tubing or connections rather than independent adjustment by anyone present.
- 8. team readiness.** Turn 17. Team readiness means the required staff and preparations are actually in place, not merely that the radiographer has arrived.
- 9. cleaning protocol.** Turn 19. The cleaning protocol is the approved decontamination process appropriate to the equipment and infection-control requirements.
- 10. handoff acknowledgment.** Turn 20. Handoff acknowledgment confirms receipt of the preparation plan and responsibilities instead of leaving them as an unconfirmed message.

F Transfer conversation

Page 48

1. requested

Nurse: "Transfer assistance has been requested."

A request has been made but is not proof that assistance is present.

2. arrived

Radiographer: "The colleague has not yet arrived."

The scenario explicitly says the assisting colleague has not arrived.

3. confirmed

Nurse: "Readiness is not yet confirmed."

The required assistance remains outstanding, so readiness is not established.

4. acknowledgment

Radiographer: "I will wait for the actual acknowledgment."

Explicit confirmation is required before treating the preparation as complete.

05

06 Briefing and language checks

Explaining a technical limitation

A Briefing checks / page 49

1. What limitation is identified?

A. Motion affecting the image

The supplied technical concern is motion affecting the chest image.

2. Who reviews technical adequacy?

B. Devon

Devon is the senior radiographer reviewing the technical concern.

3. What is not authorized automatically?

A. A repeat exposure

The case requires assessment rather than an automatic additional acquisition.

D Language checks / page 54

1. Which statement is precise?

C. Motion affects this image and needs review.

Describing the image's motion limitation identifies the review need without blame or an unsupported diagnosis.

2. What is not a direct patient-dose reading?

B. Exposure index

Exposure index concerns detector exposure under specified conditions rather than directly reporting individual patient dose.

3. Why review before repeating?

A. To assess adequacy and justify any additional acquisition

Review connects the additional exposure decision to clinical purpose and appropriate justification.

4. Which practice preserves integrity?

A. Retaining the image through the approved process

Preserving the image and approved record supports review and accurate quality improvement.

06 Conversation answers

Explaining a technical limitation

E Dialogue gaps

Pages 55-56

1. motion artifact. Turn 3. Motion artifact describes movement-related distortion, without assigning blame or a diagnosis.

2. diagnostic adequacy. Turn 5. Diagnostic adequacy concerns the clinical question, not a visually perfect picture.

3. anatomical coverage. Turn 7. Anatomical coverage identifies whether the required body area is included.

4. exposure index. Turn 9. Exposure index concerns detector exposure, not a direct patient-dose measurement.

5. repeat exposure. Turn 12. A repeat exposure needs appropriate review and justification, not automatic repetition.

6. Quality assurance. Turn 14. Quality assurance checks and improves the process and should use accurate observations rather than unsupported judgments about the patient.

7. technical review. Turn 16. Technical review assesses acquisition and image-quality features and can identify improvements without turning them into a diagnosis.

8. Reject analysis. Turn 18. Reject analysis reviews rejected images for improvement and depends on an accurate retained record rather than concealed evidence.

9. additional acquisition. Turn 19. An additional acquisition collects further image data and must be based on the appropriate reviewed decision rather than assumed permission.

10. technical adequacy. Turn 20. Technical adequacy concerns the image's technical fitness for purpose and must be distinguished from a diagnostic interpretation.

F Transfer conversation

Page 57

1. incomplete

Radiographer: "The anatomical coverage is incomplete."

The required area is not fully included in the supplied image.

2. available

Reviewer: "The image must remain available."

The scenario explicitly preserves the image for appropriate review.

3. decided

Radiographer: "Additional acquisition is not yet decided."

The reviewer has not yet made the additional-acquisition decision.

4. assessment

Reviewer: "We need the appropriate assessment."

The decision requires the actual assessment rather than an automatic repeat.

06

07 Briefing and language checks

Responding to requests for results

A Briefing checks / page 58

1. What does Jules ask about?

D. Whether a mark means cancer

Jules asks an anxious question but no diagnostic interpretation is supplied.

2. Who interprets the images?

D. The reporting clinician

The reporting clinician is responsible for the diagnostic interpretation in this case.

3. What is not supplied?

D. A reporting deadline

No deadline is given, so Ren must not invent a result time.

D Language checks / page 63

1. Which response is most helpful?

D. I cannot interpret that, but I can confirm the reporting route.

The response combines an accurate professional boundary with a concrete next step.

2. Which distinction matters?

B. Report access and clinical explanation

A patient may access a report before its meaning has been explained in context.

3. What timing claim is unsupported?

C. The result will definitely be ready tomorrow.

The case provides no reporting deadline, so tomorrow would be an invented promise.

4. What does clinical correlation mean?

B. Interpreting a finding with other clinical information

Clinical correlation places the image finding within the wider clinical evidence.

07 Conversation answers

Responding to requests for results

E Dialogue gaps

Pages 64-65

1. unverified impression. Turn 4. An unverified impression is not a reliable diagnosis or reassurance.

2. reporting clinician. Turn 6. The reporting clinician interprets images; acquiring them is a different responsibility.

3. Result communication. Turn 8. Result communication includes explanation, not merely a report existing in the system.

4. patient portal. Turn 10. Patient-portal access does not establish that the report has been clinically explained.

5. Clinical correlation. Turn 12. Clinical correlation interprets imaging alongside the patient's other relevant clinical information.

6. Comparison images. Turn 14. Comparison images are earlier examinations used to assess differences and may provide context for the reporting clinician.

7. report turnaround. Turn 16. Report turnaround is the actual time to report availability and cannot be invented as a guaranteed deadline in this case.

8. follow-up contact. Turn 18. A follow-up contact gives the patient a concrete person or service for subsequent communication rather than a vague instruction to wait.

9. scope of practice. Turn 20. Scope of practice defines the duties a professional is authorized and competent to perform, including the boundary around diagnostic interpretation.

10. result inquiry. Turn 20. A result inquiry is a request about examination findings and deserves an appropriate communication route rather than unsupported speculation.

F Transfer conversation

Page 66

1. portal

Pat: "The report is visible in the portal."

The report is available through the online patient portal.

2. explanation

Radiographer: "Access is not the same as clinical explanation."

Seeing the report does not mean its significance has been discussed.

3. contact

Pat: "I need the appropriate contact."

The next communication step is identifying the relevant clinician or service.

4. here

Radiographer: "No follow-up decision is supplied here."

The exercise provides access information but no clinical follow-up decision.

08 Briefing and language checks

Reporting a labeling near miss

A Briefing checks / page 67

1. What does Kim notice?

B. A possible label mismatch

The case supplies a possible mismatch without establishing cause or downstream impact.

2. What happens to the original record?

C. It is preserved

Preserving the original record supports the authorized review and correction process.

3. What remains unestablished?

A. How the mismatch arose

The cause and any downstream impact have not yet been established.

D Language checks / page 72

1. Which opening is factual?

B. The label may not match verified details.

The wording reports the observed concern without inventing cause or impact.

2. What must remain traceable?

A. The original record and authorized changes

An audit trail preserves the history needed to understand and verify a correction.

3. Which conclusion is premature?

C. No downstream recipient was affected.

The case explicitly leaves downstream impact unverified, so absence of impact cannot be asserted.

4. What is system reconciliation for?

C. Aligning related records across affected systems

Reconciliation checks related records so a local correction is not mistaken for complete resolution.

08 Conversation answers

Reporting a labeling near miss

E Dialogue gaps

Pages 73-74

1. label discrepancy. Turn 3. A label discrepancy is a conflict between the image label and verified examination information and should be described before its cause is assumed.

2. release hold. Turn 5. A release hold temporarily prevents further distribution while the relevant concern is reviewed through the approved process.

3. event chronology. Turn 7. An event chronology records the sequence and timing of known actions, which helps review without turning guesses into facts.

4. preserved original. Turn 9. A preserved original retains the initial information through the approved process so the review and any correction remain traceable.

5. Unconfirmed impact. Turn 12. Unconfirmed impact means possible consequences have not yet been established and cannot be described as definitely absent.

6. downstream recipient. Turn 13. A downstream recipient is a later person or system receiving the information and may require appropriate notification or correction.

7. correction authorization. Turn 15. Correction authorization is the required approval for amending the relevant record rather than an informal independent alteration.

8. audit trail. Turn 16. An audit trail preserves the history of actions and changes so a correction can be understood and verified later.

9. root cause. Turn 18. A root cause is an underlying factor supported by investigation, not an assumption made from the first visible discrepancy.

10. record integrity. Turn 20. Record integrity is the accuracy and trustworthiness of the maintained record, including traceable corrections and unresolved facts.

F Transfer conversation

Page 75

1. complete

Reporter: "The local correction is complete."

The supplied facts confirm completion only in the local record.

2. unchecked

Supervisor: "The linked system remains unchecked."

The scenario explicitly leaves the linked system unverified.

3. retained

Reporter: "The original and history are retained."

The record and change history remain available for traceability.

4. confirmed

Supervisor: "Full reconciliation is not yet confirmed."

One unchecked linked system prevents claiming that reconciliation is complete.

09 Conversation answers

Positioning instructions through an interpreter

E Extended conversation

Pages 76-77

1. single-step instruction. Turn 3. A single-step instruction asks for one action at a time and allows the patient to understand and respond before another action is added.

2. patient response. Turn 7. The patient response communicates the person's understanding, ability, or concern and must be interpreted rather than supplied by the interpreter.

3. understanding check. Turn 12. An understanding check asks the patient to demonstrate or restate the meaning and verifies the explanation rather than the interpreter's knowledge.

4. pause signal. Turn 16. A pause signal provides a clear way for the patient to communicate a need to stop or clarify within the actual examination process.

5. informed agreement. Turn 18. Informed agreement requires the relevant understanding and authorization rather than being inferred solely from silence or a nod during interpretation.

6. role boundaries. Turn 20. Role boundaries keep clinical assessment and authorization with the appropriate professionals while the interpreter supports accurate communication.

F Transfer exchange

Page 78

1. instructions

Interpreter: "Please separate the two instructions."

The interpreter asks for the combined movement directions to be given separately.

2. movement

Radiographer: "I will give one movement at a time."

Each instruction will now address one movement rather than two together.

3. agreed

Interpreter: "The patient has not yet agreed."

The scenario states that agreement to the movement is not yet established.

4. proceeding

Radiographer: "We will check understanding before proceeding."

The preparation requires a clear explanation and understanding before proceeding.

10 Conversation answers

The imaging worklist is unavailable

E Extended conversation

Pages 79-80

- 1. system outage.** Turn 3. A system outage is loss of access or function in the relevant system and requires the approved continuity process rather than informal workarounds.
- 2. patient matching.** Turn 6. Patient matching links the correct person with the correct request and record and must remain reliable during downtime.
- 3. downtime procedure.** Turn 8. The downtime procedure is the approved process for continuing appropriate work during a system interruption and must not be invented in a language exercise.
- 4. acquisition record.** Turn 10. An acquisition record documents the actual examination data and timing and is needed for accurate later reconciliation.
- 5. reconciliation ownership.** Turn 16. Reconciliation ownership identifies who will verify and align the downtime records with the restored systems instead of leaving the task unassigned.
- 6. Duplicate prevention.** Turn 18. Duplicate prevention avoids creating multiple conflicting records for the same examination and remains important both during downtime and after restoration.

F Transfer exchange

Page 81

1. restored

Technologist: "System access is restored."

The worklist is available again in the supplied update.

2. verification

Lead: "Two records still need verification."

Two downtime records have not yet been checked against the restored system.

3. complete

Technologist: "Reconciliation is not complete."

Restored access does not establish that all records have been reconciled.

4. unchecked

Lead: "Possible duplicates remain unchecked."

The scenario states that duplicate records have not yet been ruled out.

11 Conversation answers

A patient asks to stop the examination

E Extended conversation

Pages 82-83

- 1. request to stop.** Turn 4. A request to stop communicates that the person wants the current activity paused and must not be ignored as automatic consent to continue.
- 2. positioning intolerance.** Turn 6. Positioning intolerance describes difficulty tolerating a particular position and is not necessarily refusal of every possible examination or care option.
- 3. physical contact.** Turn 8. Physical contact includes hands-on assistance and requires appropriate explanation, agreement, and professional judgment rather than being assumed.
- 4. clinical review.** Turn 12. Clinical review assesses the actual situation and suitable next step rather than authorizing an improvised alternative from this conversation.
- 5. support arrangement.** Turn 16. A support arrangement identifies appropriate practical or interpersonal help within the examination's real safety and privacy requirements.
- 6. revised agreement.** Turn 20. A revised agreement concerns the next proposed step after explanation and review and must not be inferred from the earlier preparation alone.

F Transfer exchange

Page 84

1. discussion

Patient: "I agree to a discussion."

The patient agrees to discussing an alternative, not to performing it.

2. agreed

Radiographer: "Another movement is not yet agreed."

Agreement to conversation does not authorize a positioning attempt.

3. pending

Patient: "The review remains pending."

The appropriate clinical review has not yet concluded.

4. step

Radiographer: "We will explain the proposed next step."

The next proposed action requires explanation after the relevant review.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Resolving a request discrepancy

Invite the account: Please tell me which wrist was injured.

Avoid blame: I am checking the information, not asking you to resolve the paperwork.

Confirm the resolution: We will proceed once the discrepancy is appropriately resolved.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Adapting positioning instructions

Give one step: Please turn slightly toward me, then stop.

Clarify ability: Which part of that movement is difficult?

Rehearse first: We can practice the instruction before acquiring the image.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Asking sensitive screening questions

Give the reason: This question is part of our examination-screening process.

Record accurately: I will record uncertainty rather than a negative answer.

Return with a plan: We will explain the reviewed plan before proceeding.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Helping a child cooperate

Address the child: Mina, I will explain what happens before we start.

Rehearse: We can practice that instruction before taking the picture.

Agree on a pause: Let us agree how you can tell me you need a pause.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Coordinating portable imaging

Identify the request: I am here for the requested portable chest examination.

Protect connections: I will not disconnect that line or change its settings.

Close the handoff: Please acknowledge the preparation plan before we proceed.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Explaining a technical limitation

Describe the issue: There is visible motion affecting this image.

Avoid automatic repetition: A repeat needs the appropriate assessment and justification.

Support improvement: Let us identify what can improve the next instruction.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Responding to requests for results

Acknowledge the concern: I can hear that the mark is worrying you.

Avoid speculation: I cannot identify that as a diagnosis from this conversation.

Offer a concrete contact: I can help identify the appropriate contact for your question.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Reporting a labeling near miss

Lead with the discrepancy: The label may not match the verified examination details.

Keep the original: I have preserved the original record.

Close factually: I will record the confirmed actions and outstanding questions.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

accession number	verify the accession number	5
acquisition	pause image acquisition	6
adapted technique	discuss an adapted technique	15
additional acquisition	assess additional acquisition	51
age-appropriate explanation	give an age-appropriate explanation	32
alignment	check patient alignment	14
anatomical coverage	check anatomical coverage	50
anatomical landmark	identify an anatomical landmark	6
assent	seek meaningful assent	32
attached line	identify attached lines	41
audit trail	preserve the audit trail	68
bedside constraint	communicate bedside constraints	42
bedside coordination	request bedside coordination	41
bedside examination	prepare a bedside examination	41
benefit-risk discussion	arrange a benefit-risk discussion	23
bounded choice	offer a bounded choice	32
breath-hold instruction	rehearse a breath-hold instruction	15
caregiver permission	verify caregiver permission	32
caregiver presence	clarify caregiver presence	32
child-life support	request child-life support	32
cleaning protocol	follow the cleaning protocol	41
clinical correlation	explain clinical correlation	60
clinical handoff	confirm the clinical handoff	6
clinical handoff	confirm the clinical handoff	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

clinical handoff confirm the clinical handoff	24
clinical handoff confirm the clinical handoff	33
clinical handoff confirm the clinical handoff	42
clinical handoff confirm the clinical handoff	51
clinical handoff confirm the clinical handoff	60
clinical handoff confirm the clinical handoff	69
clinical indication clarify the clinical indication	5
clinical urgency communicate clinical urgency	24
collimation assess collimation	50
comparison images obtain comparison images	59
contributing factor identify contributing factors	68
cooperation support cooperation	33
coping strategy agree on a coping strategy	32
correction authorization obtain correction authorization	69
critical result escalate a critical result	59
data correction request a data correction	68
demonstration offer a demonstration	32
detector exposure review detector exposure	50
detector placement coordinate detector placement	41
diagnostic adequacy assess diagnostic adequacy	50
distress cue notice a distress cue	32
documented plan review the documented plan	6
documented plan review the documented plan	15
documented plan review the documented plan	24

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

documented plan review the documented plan	33
documented plan review the documented plan	42
documented plan review the documented plan	51
documented plan review the documented plan	60
documented plan review the documented plan	69
downstream recipient identify downstream recipients	68
equipment adjustment request authorized equipment adjustment	42
equipment clearance check equipment clearance	41
event chronology document the event chronology	68
examination protocol follow the examination protocol	23
examination site verify the examination site	5
exposure factor review exposure factors	51
exposure history clarify the exposure history	24
exposure index interpret the exposure index	50
exposure signal explain the exposure signal	33
final report communicate the final report	59
follow-up contact provide a follow-up contact	60
handoff acknowledgment obtain handoff acknowledgment	42
identity discrepancy resolve an identity discrepancy	5
image contrast assess image contrast	50
image interpretation arrange image interpretation	59
image noise assess image noise	50
image processing distinguish image processing	51
imaging request review the imaging request	5

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

immobilization explain proposed immobilization	15
incident report submit an incident report	68
incidental finding explain an incidental finding	59
infection precaution follow infection precautions	41
ionizing radiation explain ionizing radiation	23
isolation status verify isolation status	41
justification review examination justification	23
label discrepancy report a label discrepancy	68
lateral position clarify the lateral position	14
laterality verify laterality	5
learning review support a learning review	69
line management coordinate line management	42
mobile X-ray unit position the mobile X-ray unit	41
mobility limitation accommodate a mobility limitation	14
monitoring equipment protect monitoring equipment	41
motion artifact identify motion artifact	50
movement cue give a clear movement cue	15
near miss report a potential near miss	68
notification route follow the notification route	69
optimization explain optimization	23
order clarification obtain order clarification	6
oxygen delivery device identify the oxygen delivery device	41
pain-limited movement document pain-limited movement	15
patient identifier confirm patient identifiers	5

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

patient portal explain patient-portal access	60
patient preference elicit the patient preference	6
patient preference elicit the patient preference	15
patient preference elicit the patient preference	24
patient preference elicit the patient preference	33
patient preference elicit the patient preference	42
patient preference elicit the patient preference	51
patient preference elicit the patient preference	60
patient preference elicit the patient preference	69
patient support arrange patient support	42
physical assistance ask before physical assistance	15
portable radiography coordinate portable radiography	41
positioning explain the positioning	14
positioning error describe a positioning error	50
possible pregnancy report possible pregnancy	23
practice run complete a practice run	32
pre-examination check complete the pre-examination check	5
pregnancy screening explain pregnancy screening	23
preliminary report distinguish a preliminary report	59
preparation sequence simplify the preparation sequence	33
preserved original maintain the preserved original	69
privacy preference respect a privacy preference	24
private conversation offer a private conversation	23
projection clarify the requested projection	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

protective arrangement	confirm protective arrangements	32
qualified interpreter	request a qualified interpreter	6
qualified interpreter	request a qualified interpreter	15
qualified interpreter	request a qualified interpreter	24
qualified interpreter	request a qualified interpreter	33
qualified interpreter	request a qualified interpreter	42
qualified interpreter	request a qualified interpreter	51
qualified interpreter	request a qualified interpreter	60
qualified interpreter	request a qualified interpreter	69
quality assurance	support quality assurance	51
radiation exposure	discuss radiation exposure	23
radiologist review	request radiologist review	23
radiology report	review the radiology report	59
range of motion	assess available range of motion	14
reassurance	give honest reassurance	33
record integrity	protect record integrity	68
referring service	contact the referring service	59
reject analysis	conduct reject analysis	51
release hold	apply an authorized release hold	68
repeat exposure	justify a repeat exposure	50
report addendum	review a report addendum	59
report turnaround	verify report turnaround	59
reporting clinician	identify the reporting clinician	59
reproductive history	ask about reproductive history	24

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

request amendment	record a request amendment	6
requesting clinician	contact the requesting clinician	5
requisition	reconcile the requisition	5
resolved discrepancy	confirm a resolved discrepancy	6
result communication	confirm result communication	59
result inquiry	handle a result inquiry	60
review outcome	explain the review outcome	24
root cause	investigate the root cause	68
rotation	limit uncomfortable rotation	14
scope of practice	respect scope of practice	60
screening response	document the screening response	24
screening uncertainty	preserve screening uncertainty	23
sensitive history	obtain a sensitive history	23
sensory description	provide a sensory description	32
side marker	verify the side marker	5
source attribution	preserve source attribution	6
source attribution	preserve source attribution	15
source attribution	preserve source attribution	24
source attribution	preserve source attribution	33
source attribution	preserve source attribution	42
source attribution	preserve source attribution	51
source attribution	preserve source attribution	60
source attribution	preserve source attribution	69
spatial resolution	describe spatial resolution	50

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

stillness explain brief stillness	33
stop signal agree on a stop signal	33
supine position describe the supine position	14
support aid select an approved support aid	14
system reconciliation coordinate system reconciliation	69
teach-back use teach-back without blame	6
teach-back use teach-back without blame	15
teach-back use teach-back without blame	24
teach-back use teach-back without blame	33
teach-back use teach-back without blame	42
teach-back use teach-back without blame	51
teach-back use teach-back without blame	60
teach-back use teach-back without blame	69
team readiness confirm team readiness	42
technical adequacy review technical adequacy	50
technical review request a technical review	51
transfer assistance arrange transfer assistance	14
unconfirmed impact state unconfirmed impact	69
unverified impression avoid an unverified impression	60
upright position explain an upright position	14
verification pause explain a verification pause	6
verified detail distinguish verified details	68
weight bearing clarify weight-bearing ability	14
worklist review the worklist	5

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] AHRQ. Use the Teach-Back Method: Tool 5.

[Open the original source](#)

Supports checking the clarity of an explanation without treating the patient as the problem. The original conversations are language practice, not copied clinical scripts. Checked 10 October 2026.

[2] RadiologyInfo. Chest X-ray.

[Open the original source](#)

Background for plain radiography, positioning, image acquisition, and interpretation responsibilities. Checked 10 October 2026.

[3] ASRT. Practice Standards for Medical Imaging and Radiation Therapy.

[Open the original source](#)

Professional context; actual duties depend on law, competence, authorization, and setting. Checked 10 October 2026.

[4] ASRT. Position Statements.

[Open the original source](#)

Context for professional responsibilities, patient communication, image integrity, and appropriate escalation. Checked 10 October 2026.

Phil Stilwell

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